WILLIAM J. CLINTON FOUNDATION (WJCF)

Annual Report 2024-25

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Message from the Managing Director

HARKESH DABAS,
Managing Director, WJCF

It is my privilege to present the first edition of the William J. Clinton Foundation's (WJCF) Annual Report. As we mark 18 years of operations in India, we reflect on a journey defined by impact, partnership, and learning. Our mission has always been to strengthen health systems and improve lives. The achievements of this year demonstrate what can be accomplished when strong government institutions, collaborative planning, and effective public programs align with dedicated implementation on the ground.

In 2024–25, WJCF provided technical, strategic, and operational assistance across a broad spectrum of health priorities. Our support areas ranged from infectious and non-communicable diseases (NCDs) and maternal and child health to digital health and health financing, working hand in hand with government partners to maximize impact. This year truly demonstrated the power of partnership and innovation in delivering results.

Our AI-enabled chest X-ray initiative, implemented in collaboration with 13 state governments, extended life-saving diagnostic services to underserved communities—reaching over 10 lakh people. In Rajasthan, the statewide scale-up of Mission Madhuhaari across all 42 districts is ensuring access to dedicated Type 1 diabetes clinics throughout the state. Over 14 lakh health records linked to Ayushman Bharat Health Accounts (ABHA) were created in more than 4,500 facilities across 30 WJCF-supported Microsites, strengthening India's digital health ecosystem. Meanwhile, the deployment of 63 portable thermal ablation devices enabled more women to receive treatment for precancerous cervical lesions. Each of these outcomes reflects the alignment of strong program design with existing public health infrastructure across the country.

In addition to these gains, WJCF ventured into new areas where our expertise can drive meaningful impact. We have begun addressing the environmental health challenge of lead poisoning by focusing on systemic surveillance reforms. We are also intensifying our efforts against the global threat of antimicrobial resistance (AMR) by targeting priority pathogens such as *Neisseria gonorrhoeae* to close critical diagnostic gaps. Project iLIFT is pioneering an integrated approach for early chronic lung diseases diagnosis, including tuberculosis and NCDs by decentralizing screening to reach the most underserved communities.

None of this would have been possible without our close partnerships with the Ministry of Health and Family Welfare and state governments, as well as the extraordinary commitment of our teams, who bring passion and purpose to their work every day. As we look ahead, we remain committed to pushing the boundaries of what is possible in public health. We will continue to harness global expertise, community-rooted knowledge, and responsible technological innovation to strengthen health systems. Above all, we will build and nurture the trusted partnerships that enable progress, ensuring that every individual has the opportunity to live a healthy and fulfilling life.

About WJCF

The William J. Clinton Foundation (WJCF) is an Indian not-for-profit organization registered under Section 8 (previously Section 25) of the Indian Companies Act, 2013. We are committed to saving lives and reducing the burden of disease in India, while strengthening the capacity to create and sustain high-quality health systems. We achieve this by partnering with government and other stakeholders to strengthen systems and by providing technical assistance to central and state governments and partner organizations for the effective delivery of health services.

OUR VISION

A country in which everyone is able to live a healthy and fulfilling life.

OUR MISSION

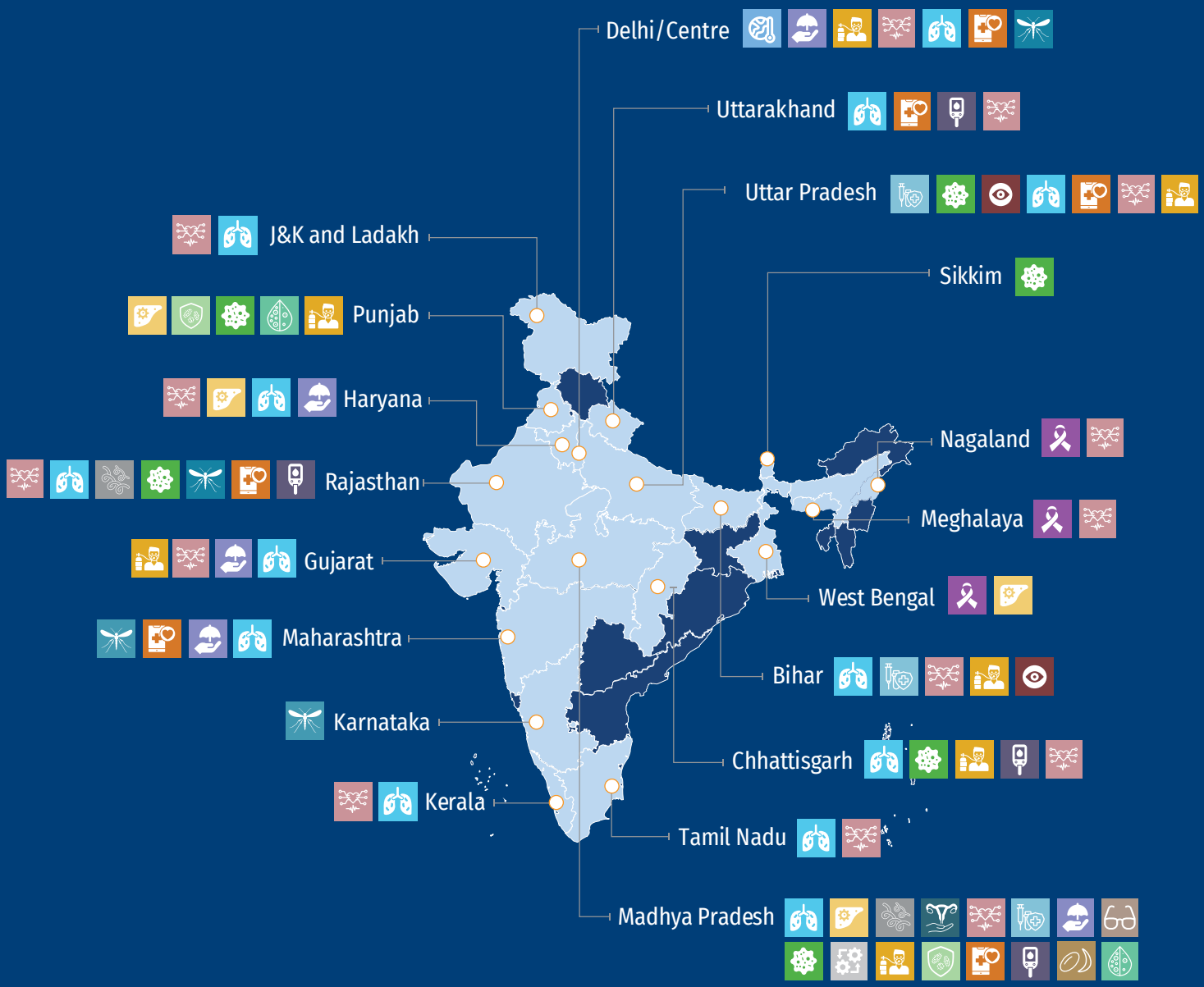
To save lives and improve health outcomes in India by enabling the government and private sector to strengthen and sustain quality health systems.

OUR VALUES

We work in cooperation with and at the service of government partners. We have an entrepreneurial and action-oriented culture which fosters diversity, equity, and inclusion. We operate with urgency, trust, transparency, frugality, and humility. We recognize our staff is our greatest asset.

Where do we work?

WJCF works under the guidance of the Ministry of Health and Family Welfare (MoHFW) and state health departments, and currently supports programs at the national level and across 20 states and union territories: Bihar, Chhattisgarh, Delhi, Gujarat, Haryana, Jammu & Kashmir, Karnataka, Kerala, Ladakh, Madhya Pradesh, Maharashtra, Meghalaya, Nagaland, Punjab, Rajasthan, Sikkim, Tamil Nadu, Uttarakhand, Uttar Pradesh, and West Bengal (Figure 1).



Key Program Areas

- AI in Health
- Anti-Microbial Resistance
- Anaemia
- Cervical Cancer
- Care Integration Services
- Climate and Health
- Digital Health
- Essential Medicines and Medical Oxygen
- Health Financing & Insurance
- Hepatitis
- HIV/AIDS
- Presbyopia
- Sexual & Reproductive Health
- Sickle Cell Disease
- Surveillance
- Syphilis
- Type 1 Diabetes
- Tuberculosis
- Vaccines
- Vector-Borne Diseases

FIGURE 1. Geographical presence of WJCF and focus disease areas across India.

Our History

WJCF was incorporated on 27 November 2007, under the Indian Companies Act, with the objective of expanding access to high-quality healthcare for vulnerable groups. Since then, our programmatic priorities have continually evolved to support government efforts in addressing the most pressing public health challenges (Figure 2).

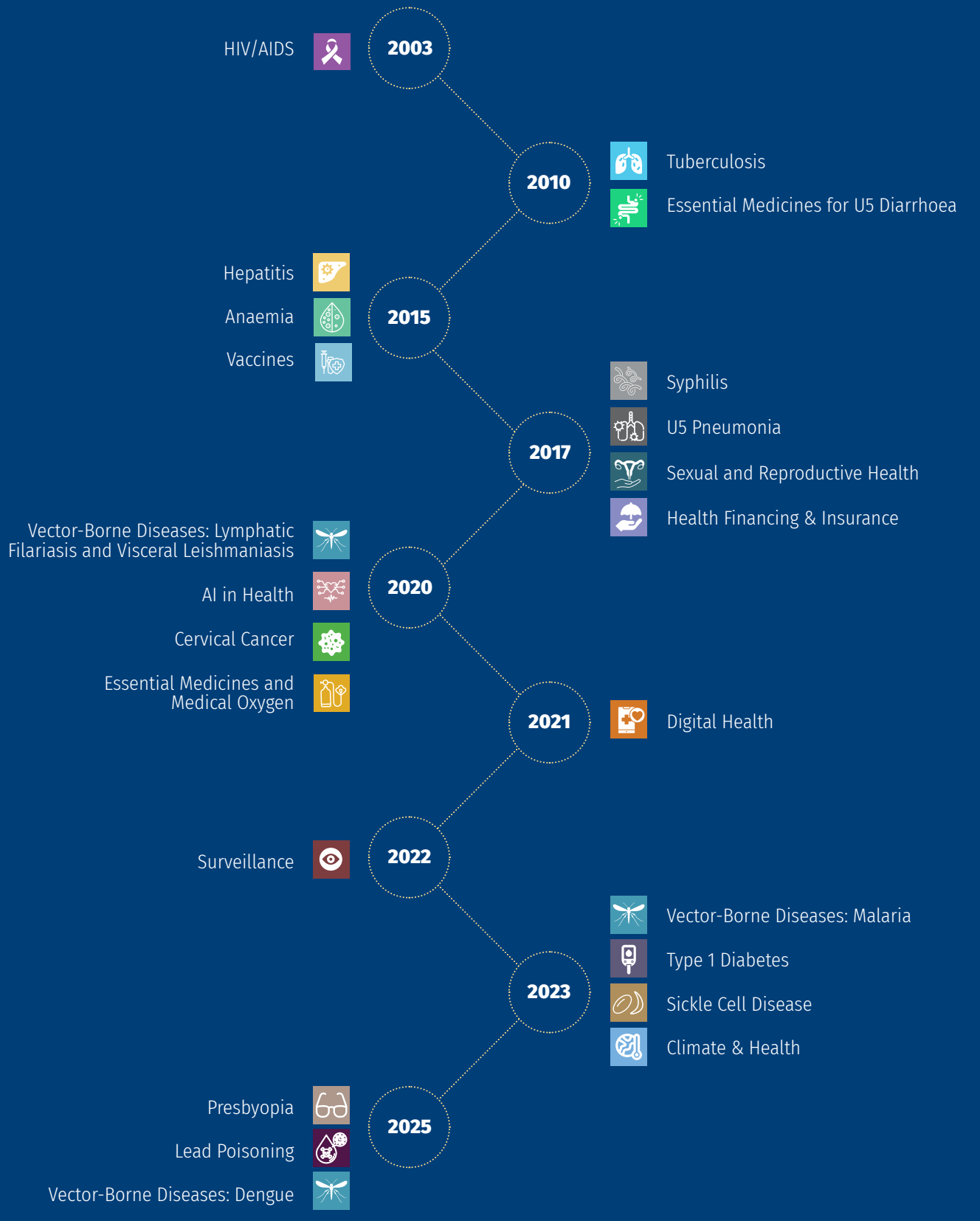


FIGURE 2. Evolution of WJCF programs.

Infectious Diseases

Advancing Tuberculosis Detection through Innovation and Inclusion

Target States/Districts:
Bihar, Delhi, Gujarat, Haryana, Kerala, Ladakh, Meghalaya, Nagaland, Rajasthan, Tamil Nadu, Uttar Pradesh, Uttarakhand

WJCF Role:
Strategic Advisor, Implementation Support, Technical Partner, Market Facilitator, Policy Engagement

Donors:
Gates Foundation; The Global Fund to Fight AIDS, Tuberculosis and Malaria (GFATM); Clinton Health Access Initiative

Partners & Collaborators:
Central TB Division (MoHFW); state governments; and implementation partners – World Vision; Centre for Health Research & Innovation; TB Alert; ALERT India; FIND for RADTECH4TB

Tuberculosis (TB) remains one of the most pressing public health challenges in India. Early symptoms like cough, fever, or fatigue are often mistaken for other illnesses, and a complete diagnosis can require multiple visits to health facilities. These delays allow TB to go undetected and spread in the community. Chest X-rays (CXR) are a vital screening tool to flag individuals who need confirmatory testing, especially in low-resource settings.

The National TB Elimination Program (NTEP) mandates CXR screening for identifying people with TB. However, access is largely limited to secondary and tertiary centres, leaving a significant gap at the primary care level, where early detection is most critical. WJCF is supporting NTEP's mission to decentralize CXR access and strengthen TB case detection by integrating AI-enabled computer-aided detection (CAD) tools into X-ray screenings. This innovation makes high-quality TB screening more timely, equitable, and accessible to key and vulnerable populations.

In its first phase, WJCF partnered with the Government of NCT of Delhi to pilot AI-enabled CXR screening at primary health centres (PHCs). Using a WHO-TDR calibration protocol, the AI screening's performance was benchmarked against radiologists' readings for ~1,411 people with presumptive TB at three PHCs. The results showed 82% sensitivity and 76% specificity. Building on this validation, WJCF developed a screening tool to identify eligible individuals (adults with chest symptoms or belonging to high-risk groups). Through this approach, 35,331 people were screened; 20% were TB suggestive on CXR; 1,839 had abnormal X-ray findings; and 193 were confirmed as having TB. Importantly, positive cases were promptly linked to government facilities for confirmatory testing. The intervention improved both accuracy and efficiency where the median turnaround time from screening to diagnosis dropped by nearly 70% (from 8 days to 2.6 days), and nearly four out of five patients received confirmatory results within a week; more than double the pre-intervention rate.

Learnings from this pilot informed a scale-up under the Global Fund's COVID-19 Response Mechanism. WJCF supported the Central TB Division (CTD) in procuring and deploying 50 ultra-portable, AI-enabled X-ray devices across 11 states. An additional 175 devices were later procured using WJCF-developed technical specifications for national scale-up.

To extend CXR screening into communities, WJCF helped operationalize 'Integrated Express Health Camps', an innovative model that combines TB screening with broader preventive health services (e.g. blood pressure, blood sugar, BMI, and, in some areas, HIV and haemoglobin testing). First launched in Tamil Nadu,

FIGURE 3: Beneficiary at the Express Health Camp in Ladakh.

FIGURE 4. Beneficiary being tested for TB using a CAD-enabled X-ray machine in Uttar Pradesh.

these camps were expanded to 10 more states, and WJCF engaged with community health workers and local influencers for mobilization, ensuring participation of key vulnerable populations and building trust in public health services. This hybrid strategy-blending technology, community engagement, and integration into government systems proved to be a scalable, high-impact approach to TB case finding (Figures 3 and 4).

Technology integration extended beyond devices to include digital systems. WJCF developed a Radiological Information System (RIS) to enable real-time, paperless workflows that link AI interpretations with laboratory results. The system supports both online and offline data entry, automates summary reporting, and features live dashboards for program monitoring. The Government of Haryana has adopted RIS for 22 devices supported through corporate social responsibility funding. Embedding AI into community health settings improved the yield of case detection nearly threefold—from about 0.3% (1 case per ~300 screenings) to 0.8% (1 case per ~127 screenings).

> “I am thankful for the chest X-ray offered in the camp. I had a persistent cough for a while and this is the first time I got a chest X-ray done.”
>
> **VIZONUO (19), BENEFICIARY,**
> KOHIMA, NAGALAND

Bringing Early Detection Closer to Women: Rakhi’s Journey in Gujarat

Rakhi, a 23-year-old woman from the Anjar block in Gujarat’s Kachchh district, exemplifies the challenges of reaching vulnerable groups through active case finding. With no X-ray facility in her block and the nearest primary health centre 12 km away, access to diagnostics was extremely limited. Prevailing sociocultural norms in her community also restrict women from leaving home without a male escort. Identified during community outreach as high-risk (she had a persistent cough for over two weeks and was a household contact of a TB patient), Rakhi initially hesitated to attend the Express Health Camp. A local Accredited Social Health Activist (ASHA) engaged with her repeatedly over several days while field teams made loudspeaker announcements to raise awareness. Eventually, Rakhi attended the camp one afternoon when her male family members were away. An AI-enabled CXR flagged her as presumptive TB, and a sputum test later confirmed the diagnosis. With continued support and counselling, she completed TB treatment and regained her health. Rakhi’s story reflects the transformative potential of doorstep health services that are inclusive, confidential, and trusted by the community.

Field information captured in February 2025, Kachchh district, Gujarat; beneficiary name changed to protect identity.

Key Accomplishments | **Active case-finding camps** (Apr 2024–Mar 2025)

Held **9,791** camps

7,88,672 individuals received CXR screenings

1,90,301 sputum samples collected

Footfall of **8,11,427** people

2,05,805 presumptive TB cases

Confirmed **6,426** TB cases

(one-third of which were asymptomatic, previously hidden infections)

Looking Ahead

The TB CXR intervention will be fully transitioned to state government ownership by December 2025. WJCF will continue to work closely with state health departments to institutionalize and sustain large-scale active case-finding services as part of the public health system. Additionally, WJCF is supporting the Central TB Division in deploying DeepCXR, India's first indigenous AI-based CXR screening tool for TB of the lungs. WJCF teams are facilitating on-ground installation, hands-on training for health staff, troubleshooting, and usage monitoring for DeepCXR while also supporting validation studies to generate evidence for future scale-up. As the tool is refined based on field performance and integrated into NTEP's Ni-kshay system, it marks a significant step forward in India's TB elimination journey. With AI-driven precision and robust partnerships, innovations like DeepCXR will help close diagnostic gaps and bring timely TB care within reach for all.

Integrating Tuberculosis Screening into High-Risk Tertiary Care Pathways

Target District:
Delhi

WJCF Role:
Strategic Advisor, Implementation Support

Donor:
Stop TB Partnership

Partners & Collaborators:
Central TB Division (MoHFW); State TB Office, Delhi; Dr. Ram Manohar Lohia Hospital; ABVIMS; VMMC; Safdarjung Hospital; Holy Family Hospital; Epitome Kidney Urology Institute

Large government hospitals often serve patients with complex comorbidities. Among them, individuals undergoing nephrology and rheumatology care face elevated risk of tuberculosis (TB) due to immunosuppressive therapies. Despite this, systematic TB screening remains under-leveraged for TB screening in overburdened outpatient departments (OPDs). To address this, WJCF launched Project FOSTER (Fostering Operational Screening, Triage, and Evaluation for Risk) to embed TB screening pathways into high-risk tertiary care services.

The project co-developed a standardized clinical algorithm tailored for outpatient departments, aligned with NTEP's Differentiated TB care guidelines. The model includes AI-enabled chest X-rays (CXR) at the point of care, rapid clinical triage, and lab-based diagnostics (e.g. Cy-TB and IGRA) for latent TB infection (LTBI). Patient navigation was introduced as a core component, ensuring those flagged as presumptive TB could move swiftly between departments, avoiding drop-offs and delays.

From April 2024 onward, four participating facilities implemented the model within nephrology and rheumatology services (Figure 5). WJCF trained medical and para-clinical staff, deployed point-of-care X-ray units, integrated workflows, and introduced a digital platform for screening and referral tracking. Nearly 14,000 patients were screened across sites. Of these, 10,500 received CXR, with 750 flagged as presumptive TB cases, leading to 225 confirmed TB diagnoses (122 clinical, 103 microbiologically confirmed). Treatment initiation stood at 100%, with early outcomes indicating ~70% treatment success.

PHOTO: WJCF

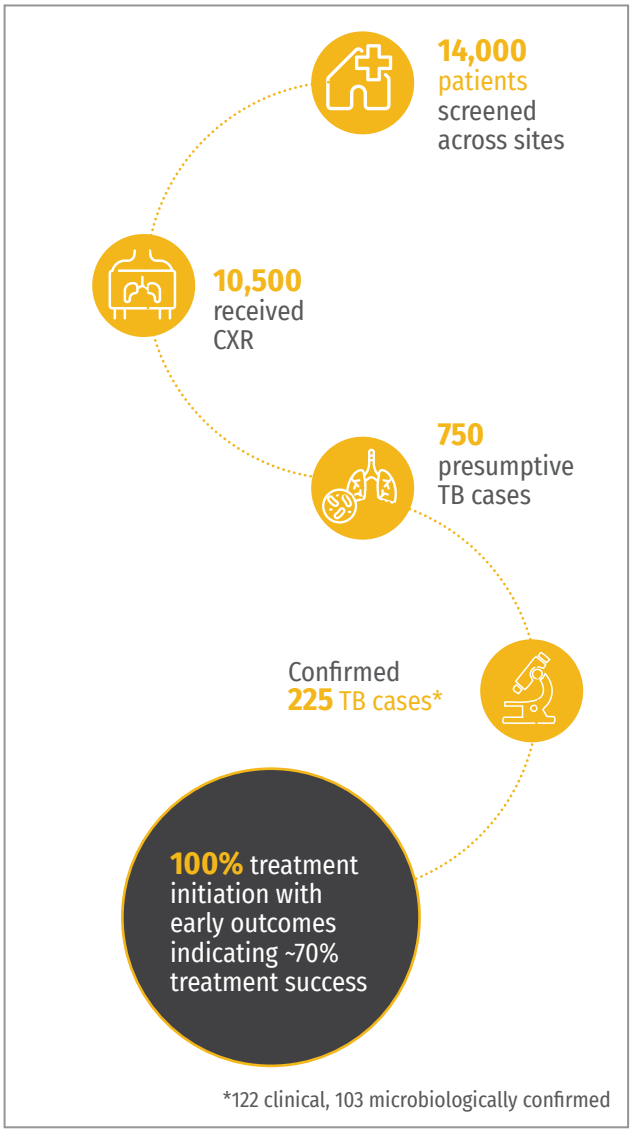
FIGURE 5. Project FOSTER field staff attending to patients in the Nephrology OPD at VMMC and Safdarjung Hospital, New Delhi.

> “Earlier, X-rays were only ordered selectively, and follow-up could take days. Now, real-time reads and on-site coordination make it easier to act quickly. This is becoming standard nephrology practice.”
>
> **SENIOR CONSULTANT, NEPHROLOGY,**
> DR. RML HOSPITAL, DELHI

To tackle the latent TB burden, 5,500+ patients underwent Cy-TB testing and 2,500 were screened using IGRA. A comparative operational validation (n=1,326) showed 79% concordance between the two assays, supporting Cy-TB as a cost-effective alternative in resource-limited settings. Of the 1,900+ individuals diagnosed with LTBI, approximately 100 were initiated on preventive therapy, marking a significant step toward TB control in immunocompromised populations.

Looking Ahead

In August 2025, the model concluded its first phase of implementation. WJCF is supporting knowledge dissemination through learning labs and evidence-sharing workshops. Insights from Project FOSTER will contribute to national tertiary care TB screening guidelines and future scale-up of differentiated TB care in India’s high-volume hospitals.



Embedding Differentiated TB Care into Public Health Systems

Target Districts:
Muzaffarpur (Bihar), Srinagar (Jammu & Kashmir), statewide support in Bihar and Haryana

WJCF Role:
Technical Partner and Implementation Support (through the TIFA Mechanism)

Donor:
John Snow, Inc (JSI)

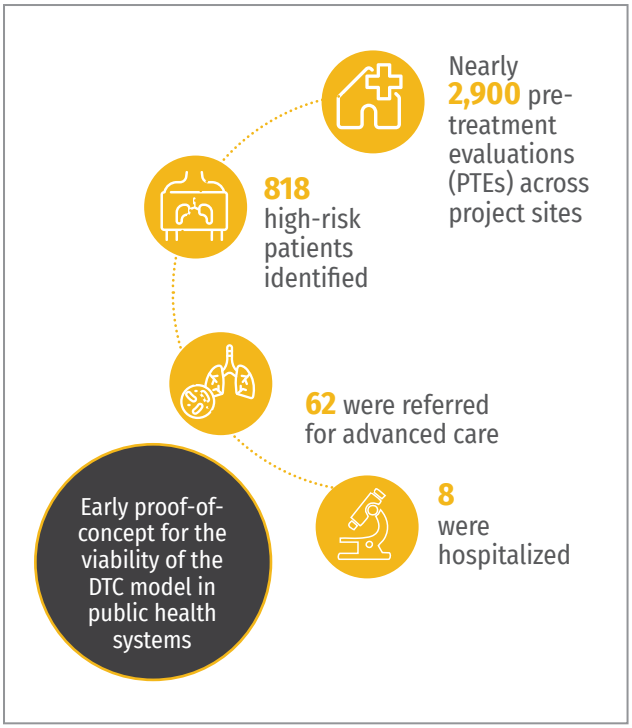
Partners & Collaborators:
Central TB Division (MoHFW); District and State TB Cells; Medical Colleges

India's TB elimination strategy increasingly recognizes that not all patients experience TB in the same way. Innovations like universal drug-susceptibility testing, digital notification, and bi-directional screening have enhanced detection and case management. However, age, comorbidities, substance use, nutritional status, and other vulnerabilities influence outcomes. To reduce TB mortality and prevent catastrophic health events, the National TB Elimination Program (NTEP) introduced the Differentiated TB Care (DTC) guidelines in 2021—an approach that stratifies patients based on risk and tailors care accordingly. Yet, for many public facilities, implementing DTC meant designing new workflows, clarifying roles, and ensuring strong clinical governance.

WJCF supported district and state TB programs in Bihar, Jammu & Kashmir, and Haryana to generate operational insights. Between December 2022 and April 2024, the team developed standard operating procedures (SOPs), referral tools, and facility workflows across 162 public health facilities, including medical colleges. District TB officers and medical officers were trained on the DTC algorithm and patient risk scoring. As of early 2024, nearly 2,900 pre-treatment evaluations (PTEs) were completed across project sites, with 818 patients identified as high-risk. Of these, 62 were referred for advanced care and eight were hospitalized, providing early proof-of-concept for the viability of the DTC model in public health systems.

In early 2024, Bihar and Jammu & Kashmir began scaling the model with WJCF support. State TB Cells integrated DTC into standard TB protocols and trained 2,100+ program personnel across 42 of 51 districts. In Muzaffarpur and Srinagar, the first two demonstration districts, WJCF supported capacity building and documentation. In Srinagar, 92% of the notified TB patients received PTEs, and in Muzaffarpur, 58%. These evaluations helped target nutritional support, screen for comorbidities like HIV and diabetes, and prioritize patients for inpatient care.

Haryana's experience added a complementary layer. WJCF supported the state government in mapping social vulnerability across 54 lakh people using a digital MIS ecosystem, including a mobile app and dashboard suite, for data collection and visualization to assess TB-related risk factors at the household level. Roughly 16.5 lakh individuals were flagged as vulnerable. This fed into a digital vulnerability index that can now be used to prioritize active case-finding, treatment adherence support, and financial protection interventions.



To integrate DTC sustainably into health systems, WJCF embedded key tools into the TB Identity Card, which is issued to all notified patients. The card now includes a DTC checklist and risk stratification matrix, prompting clinicians to assess vulnerability upfront and refer accordingly. Statewide rollout of the revised card is underway.

Timely Care Saves a Young Life in Muzaffarpur

In Muzaffarpur, Bihar, a 19-year-old woman battling tuberculosis got a new lease on life through the Differentiated TB Care (DTC) approach. She was initially diagnosed at the Comprehensive Health Centre Motipur in June 2024, but her family hesitated to follow the recommended pre-treatment evaluation. A month later, when her condition worsened, health staff assessed her under the DTC protocol and found she was severely underweight, anaemic, and at high risk (Figure 6).

PHOTO: WJCF

FIGURE 6. A woman with TB receiving specialist consultation at the Medical College.

She was quickly referred to SKMCH Medical College for specialist care, with nutritional support provided through the Nikshay Poshan Yojana. Regular follow-ups helped detect a brief treatment interruption, and timely counselling ensured she resumed medication. Today, she reports relief from symptoms and steady weight gain.

Her story shows how early risk assessment and patient-centred follow-up under the DTC initiative can save lives, even in the most challenging settings.

Field information captured between 18 June and 10 August 2024 in Motipur block, Muzaffarpur district, Bihar; name of beneficiary withheld to protect privacy.

Looking Ahead

Results and learnings from the DTC pilot were presented to the Central TB Division in January 2025. WJCF is now exploring how to support state TB programs in implementing the updated guidelines at scale, ensuring that differentiated care becomes a standard part of TB service delivery nationwide.

Using e-RUPI Vouchers to Unlock Private-Sector Chest X-rays for Presumptive TB

Target Districts:
Darbhanga (Bihar), Mumbai (Maharashtra)

WJCF Role:
Implementation Support

Donor:
Gates Foundation

Partners & Collaborators:
Central TB Division (MoHFW), Government of Bihar; Municipal Corporation of Greater Mumbai (MCGM); Gates Foundation; World Bank; Microsave; Alert India; Gurukrupa Vikas Sanstha (GVS); Doctors For You (DFY); Maharashtra Janavikas Kendra (MJK)

Nearly half of all TB patients in India first seek care in the private sector, where financial barriers are particularly pronounced, contributing to diagnostic delays. The costs, encompassing consultation fees, diagnostic procedures, and transportation, often become prohibitively high, causing interruptions or abandonment of care-seeking efforts. While the National TB Elimination Program (NTEP) offers chest X-rays (CXRs) through public facilities, these are often unavailable or difficult to access for patients under private care.

To address this, WJCF collaborated with NTEP and state partners to pilot the use of e-RUPI, a cashless, contactless digital voucher that enables patients to receive private-sector CXRs without out-of-pocket payment. This simple but powerful innovation ensures that presumptive TB patients referred by private doctors can get screened rapidly and free of cost, without needing to navigate public facility queues or reimbursements.

In Darbhanga and Mumbai, WJCF worked with the Government of Bihar and the Municipal Corporation of Greater Mumbai, and with local partners to onboard private labs, train providers, and design digital voucher workflows. More than 200 private providers were sensitized and began referring eligible patients.

Driving Financial Resilience in Diagnostics

"When payments from the paper voucher scheme haven't come for six months, how am I supposed to pay my staff?" asked Mukhtar Ansari, owner of a small diagnostic lab—Resham Diagnostics—in Mumbai, when the WJCF team approached him about participating in the e-RUPI pilot. Like many small lab owners, Mukhtar struggled with delayed payments and cash flow challenges (Figure 7).

Initially hesitant, he decided to give e-RUPI a try. By the end of the pilot, his perspective had changed completely. "I'm very happy with the instant payments enabled by e-RUPI. Patients also felt confident and well informed," he shared.

Resham Diagnostics went on to record the highest number of e-RUPI voucher redemptions in the city. The instant, transparent payments have helped Mukhtar pay his staff and suppliers on time and maintain a small emergency fund, demonstrating how digital solutions can strengthen financial resilience and efficiency in healthcare delivery.

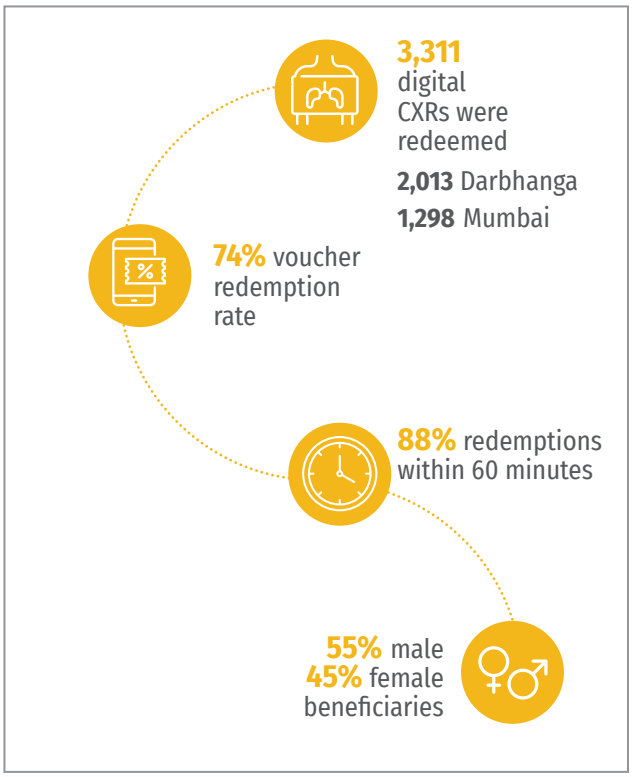
Information captured on 4 June 2024 during field interview in Dharavi, Mumbai.

PHOTO: WJCF

FIGURE 7. Resham Diagnostics lab operator being oriented on e-RUPI in Mumbai.

Between October 2023 and March 2025, 3,311 digital CXRs were redeemed: 2,013 in Darbhanga and 1,298 in Mumbai. The voucher redemption rate was 74%, and 88% of redemptions occurred within 60 minutes, indicating service access efficiency. Gender parity was notable, with 55% male and 45% female beneficiaries. E-RUPI was used by 20 participating labs to receive instant payments upon scanning the voucher, improving both turnaround times and confidence in the program.

> "I'm very happy with the instant payments enabled by e-RUPI. Patients also felt confident and well informed."
>
> **MUKHTAR ANSARI, OWNER OF A SMALL DIAGNOSTIC LAB**
> RESHAM DIAGNOSTICS IN MUMBAI



Looking Ahead

WJCF will support the development of an implementation framework for national scale-up. This will include revised SOPs, dashboards for monitoring, and tailored communication strategies for private providers. Stakeholders are also exploring the expansion of e-RUPI to cover other diagnostics, such as sputum microscopy or molecular tests, and potential applications in other public health domains.

PHOTO: SATVIR MALHOTRA

Technical Support for Lymphatic Filariasis Elimination and Visceral Leishmaniasis Sustainment

Scope:
Pan-India (Embedded TSU at National Centre for Vector-Borne Disease Control (NCVBDC), Ministry of Health & Family Welfare (MoHFW))

WJCF Role:
Technical Support Unit (TSU) for National Strategy, Surveillance, and Convergence

Donors:
Gates Foundation

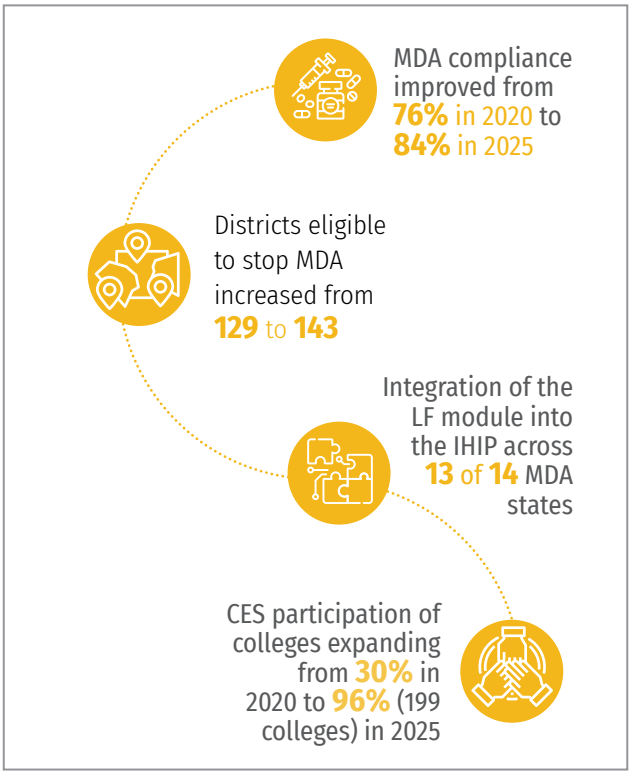
Partners & Collaborators:
National Centre for Vector-Borne Disease Control (NCVBDC), Ministry of Health & Family Welfare

India is on the cusp of eliminating two major neglected tropical diseases—lymphatic filariasis (LF) and visceral leishmaniasis (VL). VL, historically endemic to Bihar, Jharkhand, Uttar Pradesh, and West Bengal, has nearly reached the elimination threshold of fewer than one case per 10,000 at the block level. LF remains endemic to 348 districts across 20 states/union territories, with approximately 6.2 lakh lymphoedema and 1.3 lakh hydrocele cases, with elimination targeted by 2027. Both diseases disproportionately affect poor and remote communities, and eliminating them requires coordinated national planning, robust surveillance, and sustained intersectoral collaboration. Since 2020, WJCF has embedded a Technical Support Unit (TSU) within the national vector-borne disease control program to assist the Government of India in this goal.

Accelerating Lymphatic Filariasis Elimination

WJCF supported NCVBDC in designing and scaling the Block Implementation Strategy, which operationalizes LF elimination activities at the sub-district level across 1,994 blocks under MoHFW's five-pronged LF elimination plan. The team also supported the revision of the national LF guidelines released in 2024, aligning India's program with global best practices.

Through targeted government interventions, Mass Drug Administration (MDA) compliance improved from 76% in 2020 to 84% in 2025, while the number of districts eligible to stop MDA increased from 129 to 143. To strengthen data systems, WJCF supported the integration of the LF module into the Integrated Health Information Platform (IHIP) across 13 of 14 MDA states. The team also institutionalized Central Evaluation Surveys (CES) through medical colleges, with participation expanding from 30% of colleges in 2020 to 96% (199 colleges) in 2025.



Budgetary allocations have grown in tandem with these technical improvements. Honoraria for drug administrators increased from INR 600 to INR 3,000, enhancing worker motivation. Surveillance line items were expanded, and district budgets in high-endemic blocks rose from INR 5.4 lakh to INR 13.95 lakh, enabling more targeted interventions. The triple-drug regimen was scaled from 20 to 98 districts, marking a global best practice for interrupting transmission. India also transitioned to domestic procurement of diagnostics and drugs, including QFAT kits, ivermectin, and filarial test strips, improving sustainability and reducing costs.

Inter-ministerial convergence has been a hallmark of the strategy. WJCF supported stronger alignment with the National Rural Livelihoods Mission, and the Departments of Women and Child Development, Tribal Affairs, Youth Affairs, Agriculture, and Education to enable multi-pronged outreach and delivery. The TSU also developed a performance framework to rank districts and blocks, targeting underperforming areas for focused

MDA support, and created geospatial dashboards to drive data-led decision-making, optimize workforce deployment, and strengthen monitoring (Figure 8).

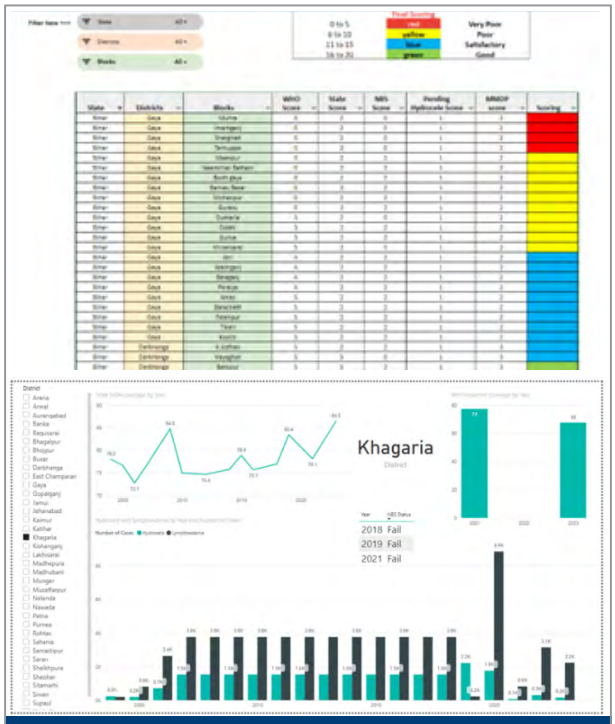


FIGURE 8. Performance and geospatial dashboards developed to drive decision-making.

Sustaining Gains against Visceral Leishmaniasis, Also Known as Kala-Azar

WJCF supported the National Kala-Azar Management Information System (KAMIS) revamp and helped transition surveillance to IHIP for full government ownership. Additionally, the team worked with 22 medical colleges to improve case detection and clinical management capacities.

Looking Ahead

By 2027, India aims to eliminate LF and consolidate its kala-azar gains. WJCF will continue supporting the elimination target across 171 districts, full transition of analytics platforms (e.g. dashboards, procurement trackers) to NCVBDC ownership, and compilation of India's elimination dossier for submission to WHO for validation.

Technical Support for Accelerating Malaria Elimination

Scope:
National

WJCF Role:
Embedded Technical Support Unit (TSU) at National Centre for Vector-Borne Disease Control (NCVBDC)

Donors:
Asia Pacific Leaders Malaria Alliance (APLMA); Gates Foundation

Partners & Collaborators:
NCVBDC, MoHFW; National Health Systems Resource Centre (NHSRC); State Vector-Borne Disease Control Programs

India has made remarkable progress in reducing its malaria burden, from nearly 20 lakh cases in 2000 to around 2.5 lakh in 2024, owing to sustained prevention, advanced vector control, and improved diagnostics and treatments. The country remains committed to eliminating malaria by 2030, with focused efforts continuing in underserved, hard-to-reach, hilly, forested, and tribal regions of Chhattisgarh, Jharkhand, Odisha, Maharashtra, Mizoram, and Tripura. Targeted interventions are helping address challenges such as treatment adherence, vector resistance, climate-related shifts, and enhanced private-sector engagement.

In 2024, WJCF established a TSU under the leadership of NCVBDC to strengthen national malaria elimination efforts. The TSU provides advocacy, technical guidance, and monitoring support across priority areas:

- **Strategic Planning:** Reviewed and finalized 27 District Action Plans (DAPs) for high-burden districts, providing technical inputs for data-driven local interventions, and operationalized a Joint Action Plan for 182 tribal districts in collaboration with the Ministry of Tribal Affairs, enabling multi-stakeholder malaria strategies.
- **Surveillance and monitoring and evaluation:** Developed dashboards and analytical reports to translate programmatic data into actionable insights, implemented a structured review mechanism across 26 low-transmission states/UTs, conducted institutional monitoring in 15 districts to identify gaps and recommend corrective actions, and supported 10 states/UTs in transitioning from paper-based to digital reporting via IHIP (Figure 9).
- **Capacity Building:** Updated and finalized five technical modules on malaria clinical management, surveillance, diagnosis, vector control, and community engagement for ASHAs, MPHWs, lab technicians, medical officers, and state program officers, ensuring alignment with the latest national malaria guidelines.

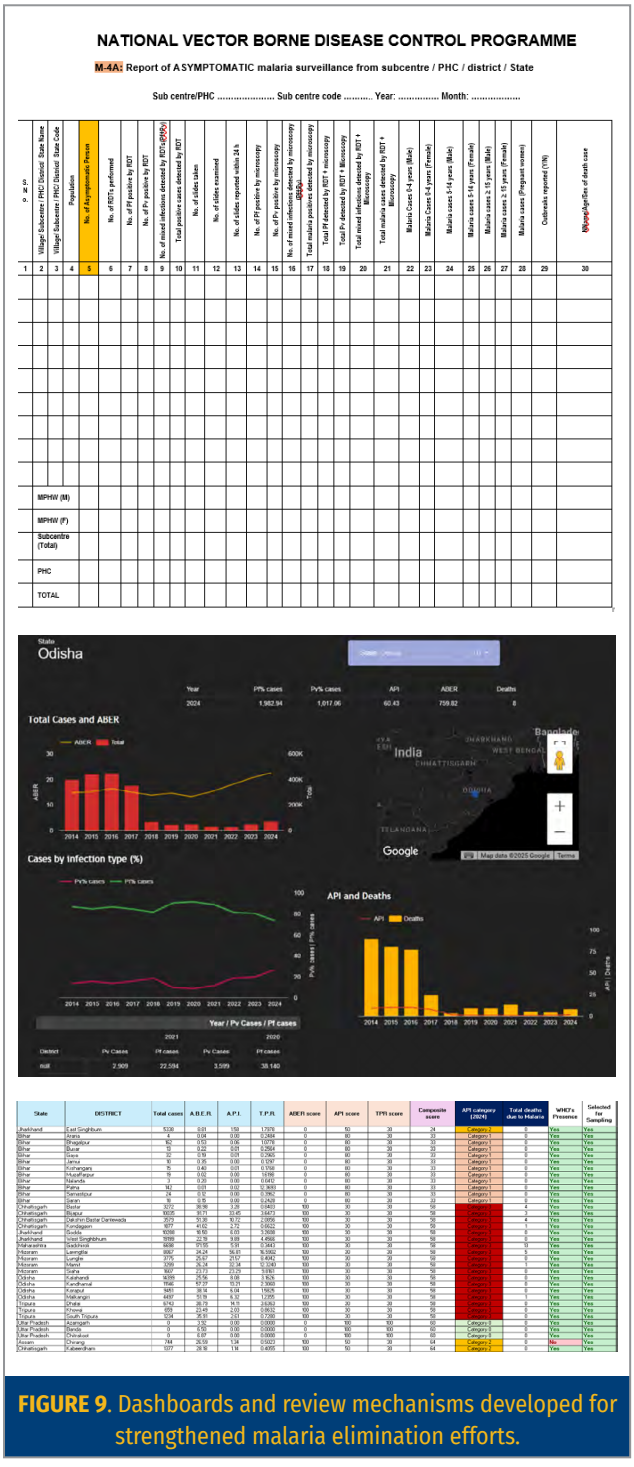


FIGURE 9. Dashboards and review mechanisms developed for strengthened malaria elimination efforts.

Turning the Tide: How Sepahijala Reversed a Malaria Outbreak in Just One Year

In 2023, Sepahijala district in Tripura reported over 500 malaria cases—a sharp rise from just 200 cumulative cases between 2016 and 2022. In response, NCVBDC, with support from WJCF's TSU, worked with state and district authorities to develop a tailored District Action Plan (DAP) with context-specific strategies to contain the outbreak.

The DAP adopted an evidence-based, participatory approach. High-burden clusters were identified through API-based stratification, followed by intensified active surveillance, an uninterrupted supply of RDT kits, and refresher trainings for ASHAs and Multi-Purpose Health Workers. Laboratory technicians were trained by the National Reference Laboratory to strengthen confirmatory diagnosis. For prevention, Long-Lasting Insecticidal Nets (LLINs) were distributed and communities sensitized on their use and maintenance.

By the end of 2024, these coordinated efforts reduced malaria cases from over 500 to just 16, lowering the district's API from 1.07 to 0.03, and enabling Sepahijala to exit the high-burden category.

Information compiled from program records and field reports for Sepahijala district, Tripura, as part of malaria outbreak response support.

Looking Ahead

WJCF TSU will continue supporting NCVBDC in implementing the Subnational Malaria Elimination Verification Framework, helping districts achieve malaria-free certification. The TSU will guide the preparation of elimination reports, support field verification, and strengthen district capacity for accurate case classification.

Additionally, in Gadchiroli, Maharashtra, WJCF will embed support within government systems to promote district ownership, enhance data-driven decision-making through IHIP, and strengthen technical support, intersectoral coordination, and community engagement to accelerate malaria elimination.

This image is for representation purpose only. Source: ertyo5/iStock

Strengthening Integrated Care to Eliminate Vertical Transmission of HIV, Syphilis and Hepatitis B

Target States/Districts:
West Bengal, Punjab

WJCF Role:
Technical Partner, Implementation Support, Policy Formulation

Donor:
Clinton Health Access Initiative

Partners & Collaborators:
WB State Viral Hepatitis Control Management Unit (SVHMU); WB State AIDS Control Society (SACS); Maternal and Child Health (MCH) Division – West Bengal; World Health Organization (WHO); USAID; Punjab SVHMU; Punjab State AIDS Control Society (PSACS); MCH Division – Punjab; other development partners; private health associations

India's maternal and newborn health agenda has been strengthened by transformational government-led efforts to eliminate the vertical transmission of HIV, syphilis, and hepatitis B virus (HBV). Expanded antenatal screening, early diagnosis, and timely treatment protocols are protecting the health of mothers and newborns across the country. In 2024, this momentum culminated in the launch of the National Guidelines for Elimination of Vertical Transmission of HIV and Syphilis (EVTHS), paving the way for an integrated nationwide push to eliminate vertical transmission.

The Triple Elimination (TE) framework expands upon EVTHS by integrating the management of HIV, syphilis, and hepatitis B, leveraging shared delivery channels for screening, counselling, and treatment through a unified maternal and newborn health platform.

Recognizing the value of convergence, WJCF supported the Government of West Bengal in establishing a dedicated State Task Force (STF) to coordinate efforts across the State AIDS Control Society, Viral Hepatitis Program, and National Health Mission. A unified TE implementation roadmap was designed by conducting comprehensive assessments at 150 sites across four pilot districts (Cooch Behar, Rampurhat, Diamond Harbour, and South 24 Parganas) and facilitating joint workshops. In December 2023, this collective effort culminated in the signing of the Kolkata Commitment, reaffirming West Bengal's pledge as one of the first states in India to adopt the TE framework at scale.

Since the pilot's launch, West Bengal, with WJCF's support, has introduced evidence-based standard operating procedures and risk-based antenatal screening while building a strong cadre of over 700 master trainers and training more than 2,000 primary healthcare facilities on TE protocols through targeted jobs aides and Information Education Communications materials to support service delivery (Figure 10). The state has also expanded syphilis treatment sites from 6 to 57, ensuring greater access to care at decentralized levels. Strengthening governance structures was a key component, with the STF now serving

FIGURE 10. A nurse showcasing job aides for labour rooms in West Bengal.

as a central coordinating body and TE reviews routinely embedded into NHM, HMIS, and district- and block-level review platforms.

Digital transformation further enhanced the program's efficiency. An integrated data system and new recording tools such as labour room slips have improved the tracking of screening, treatment, and outcomes. Routine antenatal care (ANC) screenings now facilitate early detection of co-infections among pregnant women, while behavioural risk screening during ANC identified 425 at-risk women, of whom 172 were retested in the third trimester and two new infections were detected.

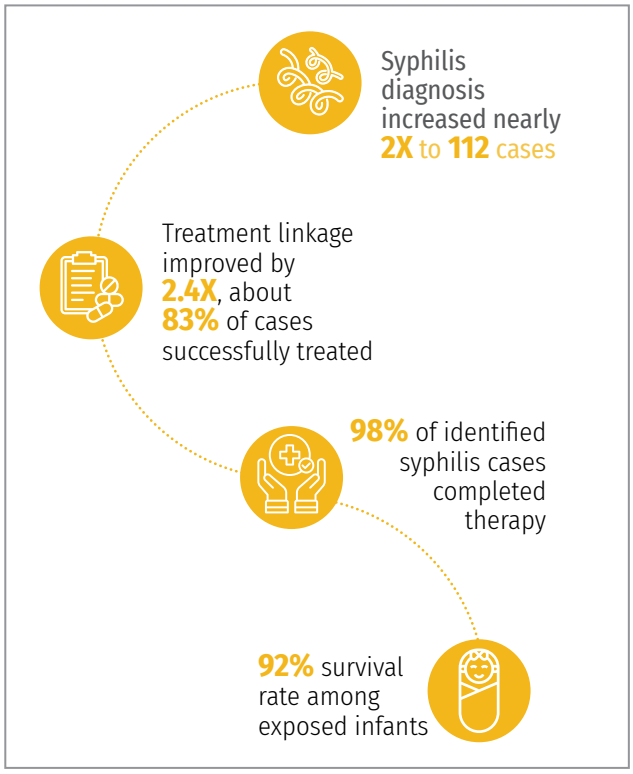
These strengthened systems and practices have translated into measurable health impact. Reported syphilis diagnosis increased nearly twofold to 112 cases, with treatment linkage improving by 2.4 times as about 83% of cases were successfully treated. Treatment adherence has also been high, with 98% of identified syphilis cases completing therapy, and a 92% survival rate among exposed infants. Preventive coverage has remained strong, as over 90% of eligible newborns received Benzathine Penicillin prophylaxis, 91% of HBV-exposed infants received both the hepatitis B birth dose and immunoglobulin, and 91% of HIV-exposed infants received antiretroviral prophylaxis.

These interventions laid the foundation for robust, integrated maternal and newborn care across all levels of the public health system.

In Punjab, WJCF supported a detailed situational assessment across 74 sites to identify key systemic challenges and inform a customized TE State Action Plan for phased implementation.

> "The healthcare workers told me about vertical transmission, and I was terrified for my child. Multiple people, including ASHA didi, reassured me and kept checking on me throughout the process. She told me that my baby would be safe if I got the complete treatment. Today, my baby does not have the disease I had, and I am very grateful for it."

AISHWARYA, A MOTHER FROM COOCH BEHAR, WEST BENGAL, WHO TESTED POSITIVE FOR SYPHILIS, REFLECTING ON HER SCREENING AND TREATMENT EXPERIENCE



Looking Ahead

Driven by the pilot's progress and learnings, the Government of West Bengal has initiated scale-up of the intervention across all 23 districts, aiming to reach over 16 lakh pregnant women and their infants annually. The state has also begun developing a 'STEM 2.0' platform (building on the pilot's digital system) to expand user access points and strengthen data-driven service delivery and program management statewide. WJCF continues to support evidence generation to inform national policy and foster adoption of the TE model across states, advancing India's commitment to eliminating the vertical transmission of HIV, syphilis, and hepatitis B.

Transforming Blood Banks into Gateways for Hepatitis Care

Target State/District:
Madhya Pradesh

WJCF Role:
Technical Partner, Implementation Support, Policy Formulation

Donor:
Clinton Health Access Initiative

Partners & Collaborators:
Madhya Pradesh State Viral Hepatitis Management Unit (SVHMU), Madhya Pradesh State Blood Cell

India has made commendable strides in strengthening its blood transfusion services. Over 1.2 crore units of blood are donated voluntarily each year, and all donations are universally screened for five major transfusion-transmitted infections (TTIs): HIV, HBV, HCV, syphilis, and malaria.

Madhya Pradesh reports an estimated hepatitis C antibody (anti-HCV) positivity of 0.08% and hepatitis B surface antigen (HBsAg) positivity of 0.5–1.0%. To address this, the State Viral Hepatitis Management Unit (SVHMU) provides free HCV and HBV testing and treatment services, including hepatitis B screening for all pregnant women as part of routine antenatal care. In blood banks, as per national guidelines, all donor blood is screened for the five TTIs. While robust follow-up systems exist for HIV and syphilis, reporting and referral mechanisms for hepatitis-positive donors remain weak, limiting timely linkage to care.

To address this, WJCF supported the development and piloting of standard operating procedures (SOPs) in seven blood banks across the state. The SOPs clearly defined the roles and responsibilities of the blood bank staff and the viral hepatitis program's staff. The intervention emphasized capacity building of key personnel, improvements in pre-donation counselling (including verifying donor contact details), and multi-pronged follow-up outreach (text messages and telephone calls) coordinated between the SVHMU and the State Blood Cell. A data management system was established to give both blood centres and treatment facilities real-time access for donor tracking and follow-up. This model demonstrated the feasibility of using blood banks as effective entry points for viral hepatitis care, supporting broader efforts under the National Viral Hepatitis Control Program (NVHCP).

Between April and September 2023, over 51,000 blood donors were screened at the pilot blood banks. Among them, 628 donors (1.2%) were reactive for HBV and 50 donors (0.1%) for HCV. Key outcomes include:

- **Linkage to care:** 57% of all sero-reactive donors were successfully referred to treatment facilities. Referral rates ranged from 50% to 98% across sites, with smaller-volume blood banks (fewer than 8,000 donations annually) achieving the highest linkage rates.
- **Efficiency gains:** Dedicated follow-up protocols reduced the median time from donation to linkage from 53 days to just 5 days—a tenfold improvement in speed.
- **Improved donor engagement:** Blood banks that completed all three follow-up call attempts within the first week post-donation saw higher appointment scheduling rates (86–100%) and significantly improved donor linkage rates (74–98%).

Based on the pilot's early success and feasibility, the Government of Madhya Pradesh has expanded the model to 15 additional blood banks statewide. This scale-up marks a significant step in operationalizing the role of blood banks in the broader hepatitis response.

Looking Ahead

WJCF will continue to support the state's vision of strengthening and sustaining this model as it expands, ensuring its long-term integration into routine services and helping more hepatitis-positive donors receive timely care.

Reproductive, Maternal, Newborn, Child, Adolescent Health + Nutrition

Advancing Sexual and Reproductive Health through Systems, Evidence, and Innovation

Target States/Districts:
Delhi, Madhya Pradesh (Chhatarpur, Guna, and Rajgarh) and Pan-India

WJCF Role:
Operational Partner, Trusted Analytical & Strategic Partner, Market Shaper

Donors:
Clinton Health Access Initiative; Gates Foundation; UNFPA; WHO

Partners & Collaborators:
Government of Madhya Pradesh; NITI Aayog; Federation of Obstetric & Gynaecological Societies of India (FOGSI); National Technical Support Unit (NTSU); Institute of Economic Growth (IEG), New Delhi; Youth organizations; Purple Audacity; Yellow Access (Mynt); Tinkerlabs

India continues to make significant strides in family planning and sexual and reproductive health (SRH). With a contraceptive prevalence rate of 66.7% and replacement-level fertility achieved, government initiatives like Mission Parivaar Vikas and the Antara injectable contraceptive program have laid a strong foundation for ensuring universal access to family planning services across the country. Looking ahead, opportunities lie in the strengthening last-mile delivery of essential commodities, expanding facility and provider readiness for long-acting methods, and enhancing counselling for informed choice. Greater emphasis on healthy birth timing and spacing, and widening the range of contraceptive options, will further align services to the diverse needs of the population—paving the way for more equitable, high-quality family planning across the country.

In 2024–25, WJCF focused on strengthening service delivery, generating evidence on the benefits of adequate birth spacing, and laying the groundwork for new contraceptive options in partnership with governments, development partners, and communities. This approach seeks to ensure access to high-quality, equitable, and user-centred SRH services.

Building Resilient Delivery Systems

WJCF partnered with the state governments of Delhi and Madhya Pradesh to strengthen equitable access to SRH services. In Delhi, under WHO's guidance, WJCF supported the design of a family planning helpline (including e-learning modules for call centre staff) to improve access to quality counselling. An operational framework was created to develop a standardized protocol for comprehensive counselling related to family planning products and services and to facilitate referrals to public facilities as required.

In Madhya Pradesh, WJCF provided technical assistance in three priority districts—Chhatarpur, Guna, and Rajgarh—where gaps in supply, infrastructure, and counselling inhibit adequate and reliable access to family planning. The team streamlined the Family Planning Logistics Management Information System (FPLMIS) by remapping over 3,000 ASHAs, training staff on indenting and stock reordering practices, and introducing performance bulletins to track commodity movement and highlight supply chain priorities. These measures enabled the resumption of contraceptive services including Antara injections, intrauterine devices (IUDs), and postpartum intrauterine contraceptive devices (PPIUCDs) across 47 previously inactive facilities.

To strengthen facility readiness for delivering high-quality family planning and maternal health services, over 630 health and wellness centres (HWCs) and 103 other health facilities spanning primary health centres, community health centres, and district hospitals were assessed in partnership with the state government. Each facility received a detailed improvement plan for

FIGURE 11. A medical officer counselling a woman on family planning.

strengthening human resources capabilities, supply chain, infection prevention, privacy, IEC materials, and demand generation.

Capacity building was a key focus: WJCF supported the training of 470 Auxiliary Nurse Midwives (ANMs) and 30 Lady Health Visitors (LHVs), covering 100% of eligible providers in the three pilot districts, along with 100 medical officers and staff nurses. These sessions strengthened informed-choice counselling, eligibility assessment for long-acting reversible contraceptives (LARCs), and side-effect management, complemented by job aids and tele-counselling support (Figure 11).

To institutionalize data-driven management, WJCF helped integrate a unified family planning dashboard that enables structured monthly reviews, facilitating evidence-based decision-making at district and block levels.

From System Lag to Supply Chain Leadership: Transforming FP Logistics in MP

Until early 2023, the districts of Chhatarpur, Guna, and Rajgarh in Madhya Pradesh faced challenges with effective supply chain management for family planning commodities (such as Chhaya pills, Mala-N, and condoms). Despite the availability of the digital Family Planning Logistics Management Information System (FPLMIS), there was limited availability of commodities due to inadequate indenting and insufficient training of frontline health workers. To address this, WJCF oriented various cadres (store in-charges, ANMs, ASHAs) on FPLMIS by integrating training into existing forums—block-level reviews, sector meetings, and one-on-one mentoring. This consistent, hands-on engagement built familiarity with the digital tool and bridged knowledge gaps.

By June 2024, all three pilot districts were consistently ranked among the top five in the state on FPLMIS performance indicators. Commodities were being indented on time, and stock issuance at HWCs and facilities became timely and streamlined.

Information compiled from Performance Bulletin and FLPMIS, Madhya Pradesh, April–June 2024.

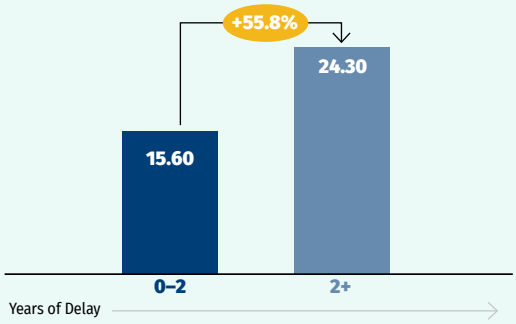
Advancing Evidence on Birth Timing and Spacing

According to National Family Health Survey-5 (NFHS-5) data, 64% of first births in India occur within the first three years of marriage, and 57% of subsequent births occur within less than two years of the previous birth. Such patterns are linked to higher risks of maternal and child morbidity and mortality, and they can constrain women's opportunities for education and work.

Greater focus on birth timing and spacing presents a powerful opportunity to improve reproductive, maternal, and child health outcomes. To inform this agenda, WJCF, with support from the Institute of Economic Growth, conducted an econometric analysis of unit-level NFHS data. The findings were compelling:

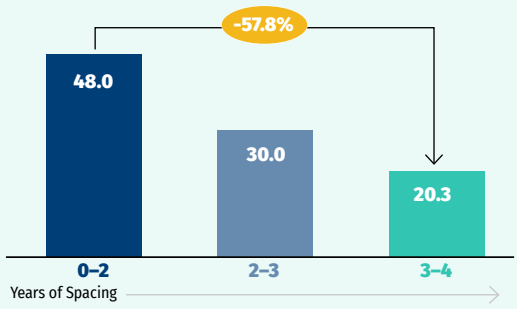
Women's workforce participation: Women who delayed their first birth by two or more years after marriage were 55% more likely to participate in the labour force (Graph 1).

GRAPH 1: Employment rate for women who had childbirth in the last five years (% ages)



Child survival: Spacing births at least three years apart reduced infant and under-five mortality by over 57% (Graph 2).

GRAPH 2: Under-five mortality per 1,000 births by years of spacing interval



Child nutrition: Children born with optimal spacing intervals were 15% less likely to be stunted and 17% less likely to be underweight (Graph 3).

GRAPH 3: Child developmental potential and correlation with spacing interval between consecutive births (% age)

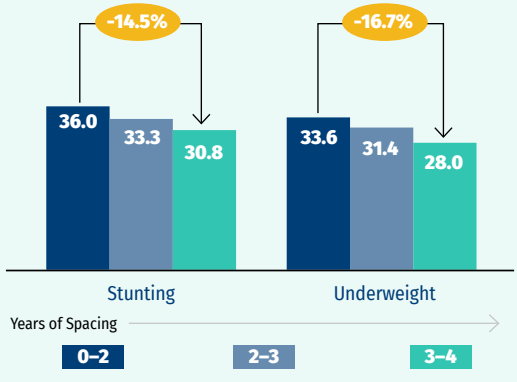


PHOTO: WJCF

FIGURE 12. Dissemination event for sharing findings of econometric analysis held in Delhi.

These findings on the health and economic benefits of optimal birth timing and spacing were disseminated at 20+ forums nationwide (Figure 12)—including the World Health Summit Regional Meeting, New Delhi in April 2025 and AICOG 2026 in January 2026—to policymakers, clinicians, program leaders, and researchers.

Expanding Contraceptive Choices

India's family planning ecosystem is evolving to meet diverse contraceptive needs, particularly among women seeking discreet, reliable, and self-controlled options. In 2024–25, WJCF advanced work on two underutilized but high-impact contraceptive methods: On-Demand Contraceptive Pills (ODCP) and hormonal implants.

ODCP is a peri-coital oral contraceptive that holds promise for women with infrequent sexual activity or an unmet need for flexible contraception. It is not yet commercially available and is currently undergoing clinical trials. To assess its potential introduction pathway, we conducted a comprehensive multi-stakeholder assessment to evaluate market readiness, forecast demand, and define the product's value proposition. This included an examination of regulatory pathways and delivery models, development of a product concept, and identification of priority user segments through consultations with healthcare providers and other key stakeholders. The resulting insights are intended to inform future planning and decision-making when ODCP is approved in India, highlighting opportunities to expand contraceptive choice while ensuring safety, affordability, and alignment with user needs. Hormonal implants are a long-acting, reversible contraceptive method introduced by the Government of India in 2023. WJCF estimated demand and assessed supply security across multiple scenarios (including imports and local manufacturing). Market landscaping, informed by stakeholder consultations and insights from frontline workers across five states (Assam, Bihar, Gujarat, Karnataka, and Rajasthan), identified key enablers and barriers to scaling implant access.

Together, these efforts have strengthened the foundation for sustainable market introduction and equitable access to modern contraceptives that meet users' needs and preferences.

Looking Ahead

WJCF remains committed to supporting ecosystem efforts that advance equitable access to sexual and reproductive health services in alignment with national priorities, including the timing and spacing of pregnancies. Future work will focus on strengthening health systems through capacity building, supply chain optimization, and institutionalizing tools for sustainability. We will continue to translate evidence into actionable strategies to support program decisions, while addressing barriers to uptake through consumer-focused interventions and innovative delivery channels, including digital platforms.

Expanding method choice will be central, with efforts to shape markets for newer options such as ODCP and implants, alongside regulatory preparedness and procurement planning. These initiatives aim to reinforce system efficiency, uphold reproductive rights, and ensure that family planning remains integral to women's health and well-being.

Improving and Sustaining Full Immunization Coverage

Target States/Districts:

Strengthening Data & Review Mechanisms: 42 districts (108 Aspirational Blocks) in Uttar Pradesh; 7 districts in Bihar

Zero-Dose Learning Agenda: East Champaran and West Champaran, Bihar; Maharajganj and Lakhimpur Kheri, Uttar Pradesh

Strengthening Urban Immunization: Jhansi and Sambhal, Uttar Pradesh; Chhindwara and Ujjain, Madhya Pradesh

WJCF Role:
Strategic Advisor, Implementation Partner, Policy Engagement, Tool Development, Adaptive Learning Implementation

Donor:
Clinton Health Access Initiative/Gates Foundation

Partners & Collaborators:
Governments of Bihar, Madhya Pradesh, and Uttar Pradesh; Uttar Pradesh Technical Support Unit (UPTSU)

Immunization is one of the most impactful public health interventions, having saved an estimated 15.4 crore lives globally in the last 50 years. India's Universal Immunization Programme (UIP), the largest of its kind, reaches 2.67 crore infants and 2.9 crore pregnant women annually, contributing to steady declines in vaccine-preventable diseases such as diphtheria, tetanus, pertussis, and measles. However, systemic and equity-related challenges persist, especially among underserved and vulnerable populations.

Since 2020, WJCF has partnered with state governments, key stakeholders, and communities across Bihar, Madhya Pradesh, and Uttar Pradesh—home to over one crore births per year—to improve Full Immunization Coverage (FIC), reduce zero-dose prevalence, and strengthen immunization systems. In 2024–25, our support focused on removing access barriers to immunization services and ensuring that progress is sustained and equitable.

WJCF supported in embedding robust governance processes and digital tools to institutionalize continuous improvement, providing intensive support in 42 districts

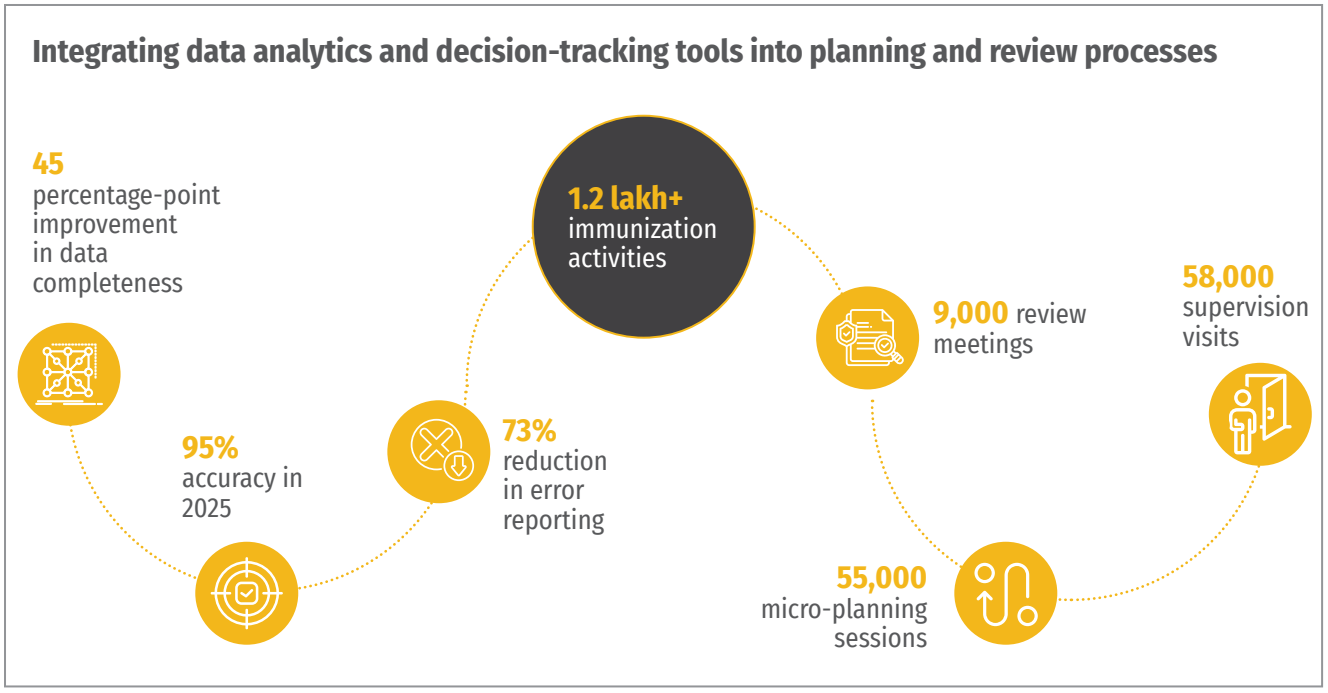
PHOTO: SATVIR MALHOTRA

FIGURE 13. Frontline health workers — the backbone of the public health systems.

(108 Aspirational Blocks) of Uttar Pradesh and seven districts of Bihar. This included equipping officials with data analytics and decision-tracking tools, integrating these tools into planning and review processes, and building the capacity of program managers and health workers to use data for planning and problem-solving (Figure 13).

The intervention led to a 45-percentage-point improvement in data completeness, achieving more than 95% accuracy in 2025, and reduced reporting errors by 73%. Over 1.2 lakh immunization activities, including 9,000 review meetings, 55,000 micro-planning sessions, and 58,000 supervision visits, were supported through digital tools on dashboard use and visualization that enabled faster feedback loops and evidence-based decision-making (Figure 14).

FIGURE 14. Building the capacity of healthcare workers on digital tools for stronger immunization outcomes.



Under the Zero-Dose Learning Agenda, efforts focused on uncovering and addressing barriers among zero-dose households in East Champaran and West Champaran in Bihar and Maharajganj and Lakhimpur Kheri in UP. WJCF used innovative methods—human-centred design surveys, machine learning-based archetype modelling, and GIS-based prioritization—to better understand why certain children remain unvaccinated. These approaches helped identify previously undiscovered zero-dose children, reveal the reasons behind non-vaccination, and design interventions to help frontline workers reach them. In collaboration with government officials, design experts, and community members, WJCF co-developed and tested job aids such as the routine immunization (RI) Flipbook, Shishu Suraksha (Child Protection) Card, Myth-Busting Cards, Swasthya Suvidha Card, and personalized session calendars (Figure 15).

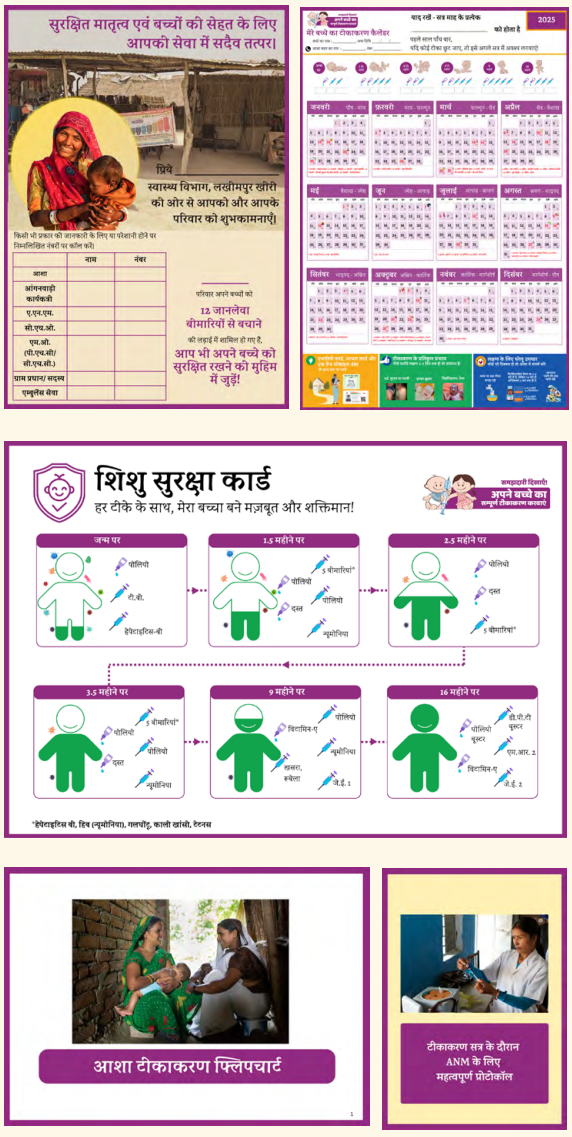


FIGURE 15. Jobs aides developed to support frontline workers reach zero-dose children.

More than 22,500 caregivers were reached, and over 1,300 ASHAs and 120 ANMs were equipped with tailored materials. Continuous monitoring, through surveys with 1,200 caregivers and 520 ASHAs, and observation of 48 session sites enabled iterative refinement and ensured scalability through government systems.

Pramila's Transformation: A Flipbook That Empowered Communication

Pramila Devi, an ASHA in Ward 7 under HSC Ghiwadhar, Harsiddhi block, East Champaran, struggled to explain vaccination benefits without visual aids. The RI Flipbook changed everything.

Using its simple pictures and messages during home visits, she now confidently explains how vaccines protect children and helps parents manage mild side effects. Mothers are better informed about handling adverse events, while Pramila's own knowledge has deepened beyond just vaccination schedules.

Caregivers now recognize vaccines better, recall flipbook information, and ask informed questions. Their ability to manage common side effects at home has significantly reduced hesitation.

The flipbook truly empowered Pramila to bridge communication gaps and serve her community effectively.

Field information captured in March 2025 at Harsiddhi block, East Champaran district, Bihar; caregiver name changed to protect identity.

> "Due to the new tools, in a short span we were able to reach and vaccinate many children who had missed doses or hadn't been vaccinated at all. That's something we couldn't do before with such clarity and focus."
>
> **DR. ARIF SIDDIQUE,** RAJEPUR BLOCK, FARRUKHABAD DISTRICT, UTTAR PRADESH

Urban immunization poses unique challenges of population mobility, private-sector engagement, and data fragmentation. To address these, WJCF worked with governments in Jhansi, Sambhal, Chhindwara, and Ujjain to improve governance, data validation, and service delivery in urban populations, especially those in vulnerable settlements with large coverage gaps.

The program supported area demarcation and session planning across four cities, leading to a 15% increase in planned sessions. Private facilities were integrated into the immunization ecosystem, expanding the reach among urban and peri-urban populations. In Madhya Pradesh, the introduction of 'U-WIN Days' accelerated registration and record updating under the U-WIN digital platform, achieving near-100% registration of eligible children and updating past records for over 90% of them.

Capacity building remained a cornerstone of the approach. Over 95% of ANMs completed all modules of the Routine Immunization Skill Enhancement (RISE) e-learning program while 218 healthcare workers were trained and mentored on urban immunization planning and service delivery, strengthening both service quality and ownership at the facility level.

> "U-WIN Days provided an impetus to the rollout of the U-WIN digital immunization platform in Madhya Pradesh. Chhindwara and Ujjain have become exemplar cities and given us a replicable model. WJCF extended the 'U-WIN Day' initiative statewide, leading to swift registration of beneficiaries and rapid updating of vaccination records in many districts."
>
> **DR. SAURABH PUROHIT, DEPUTY DIRECTOR (ROUTINE IMMUNIZATION)**, AND **MR. VIPIN SHRIVASTAVA, DEPUTY DIRECTOR (COLD CHAIN),** GOVERNMENT OF MADHYA PRADESH

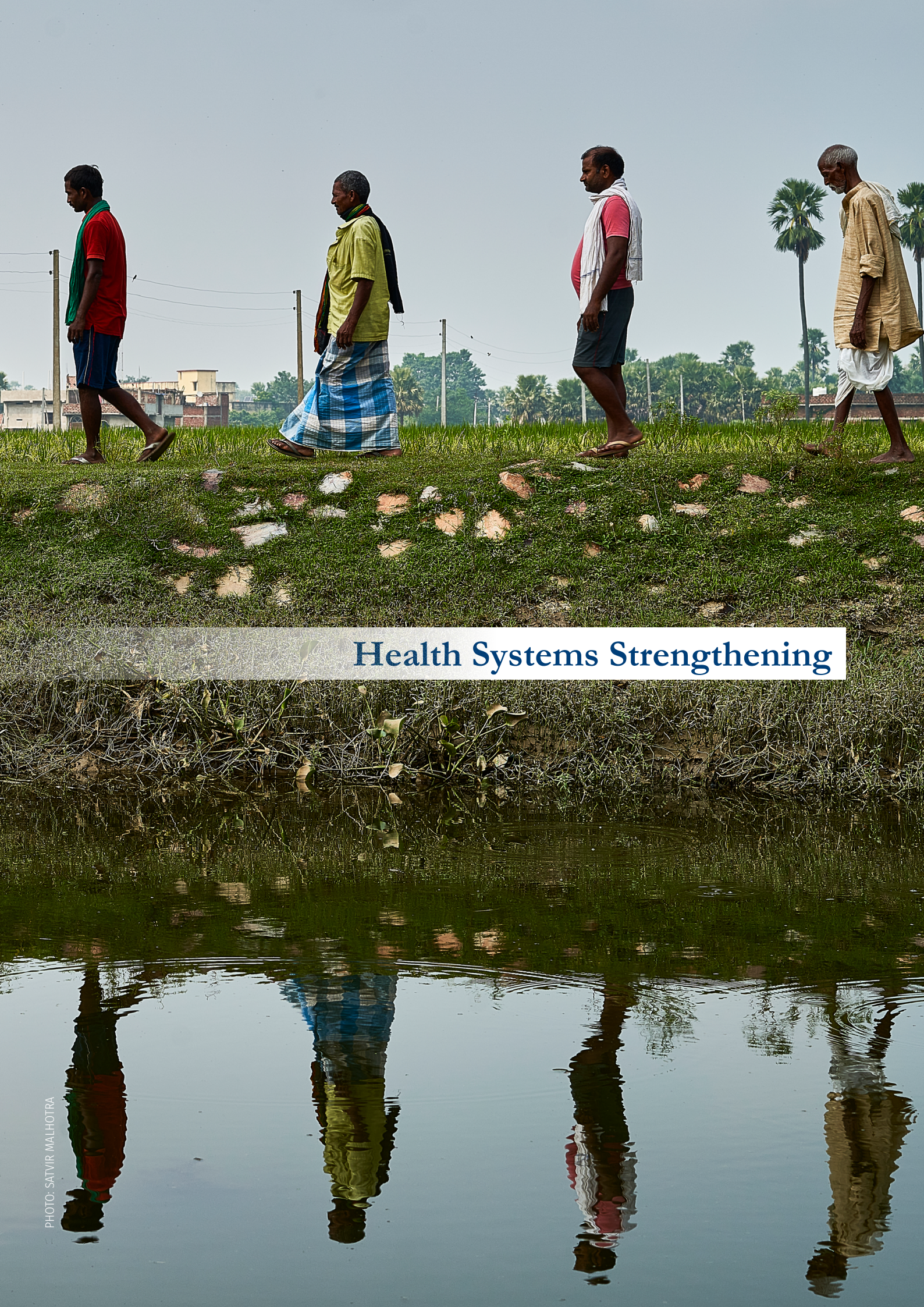
Improving the Immunization Supply Chain in Jhansi

In Jhansi, Uttar Pradesh, WJCF supported the government in activating the Alternate Vaccine Delivery (AVD) system for 250+ immunization sites. According to the AVD guidelines under the UIP, vaccines must be transported to outreach sites in a carrier box by a designated porter—ensuring cold-chain integrity and freeing the ANM to focus on service delivery. WJCF worked with Jhansi's urban health unit to kickstart vaccine deliveries, including identifying couriers, planning delivery routes, and training the personnel. This initiative eased ANMs' workload of carrying vaccines during outreach sessions and improved service delivery. (This model has since been integrated into routine practice in Jhansi.)

Information compiled from program records and field reports for urban areas of Jhansi district, Uttar Pradesh, October 2024.

Looking Ahead

Building on this year's successes, WJCF remains committed to advancing equitable, resilient, data-driven immunization systems, with an emphasis on consolidation and scale-up. We will work closely with our partner state governments to integrate proven, cost-effective, and operationally feasible solutions into routine immunization programs. This includes supporting the scale-up of the zero-dose intervention package across all 75 districts of Uttar Pradesh, with strategic and technical guidance to ensure smooth adoption. In parallel, WJCF will transition the developed data analytics and decision-tracking tools to the three state governments, advocate for their replication in other regions, and provide on-demand support to extend these tools to additional states. Our focus will be to cement these innovations into the public system so that immunization gains are sustained and no child is left behind.



Health Systems Strengthening

PHOTO: SATVIR MALHOTRA

Enabling Ayushman Bharat Digital Mission (ABDM) Adoption

Target States/Districts:
National (NHA) and 27 districts across 5 states (Madhya Pradesh, Maharashtra, Rajasthan, Uttar Pradesh, Uttarakhand)

WJCF Role:
Implementation Support, Strategic Advisor, Technical Assistance to National Health Authority (NHA)

Donor:
Gates Foundation

Partners & Collaborators:
National Health Authority; State ABDM offices – Madhya Pradesh, Maharashtra, Rajasthan, Uttarakhand, Uttar Pradesh

India's healthcare landscape has a diverse mix of public and private service providers, offering a wide range of preventive, curative, and palliative care. To bridge gaps in care continuity and to enable patient-owned health data, the Government of India launched the Ayushman Bharat Digital Mission (ABDM) in 2021 under the National Health Authority's leadership. ABDM aims to power India's quest for universal health coverage by creating an interoperable digital health ecosystem.

ABDM has made remarkable progress onboarding public health facilities and professionals, but increasing private-sector participation remains a key opportunity. While nearly two-thirds of India's doctors work in the private sector, over 63% of professionals registered so far on the National Health Professionals Registry (NHPR) are from the public sector. Similarly, of the ~4,17,925 facilities registered on the Health Facility Registry (HFR), 55% are public facilities. Key building blocks of ABDM—such as the Ayushman Bharat Health Account (ABHA) and the NHPR—enable the creation of a national health identity and access to a verified pool of providers. ABDM-enabled digital solutions also facilitate a range of consent-driven services, from efficient queue management to linking longitudinal health records to a patient's ABHA.

WJCF has partnered with the NHA since 2021, supporting state-level ABDM adoption and informing national strategy through technical assistance, field engagement, and ecosystem building. A major milestone was achieved in FY 2023–24 with a successful pilot in Gujarat that validated approaches for private-sector onboarding and informed national scale-up. The NHA incorporated learnings from this pilot to develop guidelines and toolkits for implementing 100 'Microsites' (clusters of small-to-medium private facilities in a given area) across the country under its 100 Microsites initiative.

Building on these insights, in 2024–25, WJCF deepened its engagement by supporting the NHA's 100 Microsites initiative across six states, aiming to rapidly scale ABDM adoption among private clinics, hospitals, laboratories, and pharmacies. Working closely with state ABDM offices, WJCF provided direct implementation support in 35 districts, helping accelerate private-sector participation, strengthen digital readiness, and generate early evidence on effective models for scale. The work focused on onboarding health facilities and professionals onto ABDM platforms, facilitating health record creation, and providing structured feedback to the NHA for ongoing policy refinement (Figure 16).

Field officers trained by WJCF registered more than 20,000 healthcare professionals on the NHPR—nearly

FIGURE 16. An ABDM-onboarded private health facility showcasing how to create ABHAs.

8,000 in Uttar Pradesh, 5,400 in Madhya Pradesh, 1,600 in Maharashtra, 3,900 in Uttarakhand, and 1,200 in Rajasthan. Over 15,000 facilities were onboarded onto the HFR, including approximately 9,400 clinics, 3,500 hospitals, 2,000 pharmacies, and 1,200 laboratories. Collectively, these facilities generated 14 lakh ABHA-linked health records, creating a tangible foundation for patient-controlled longitudinal data. The initiative also served as an important bridge between national policy design and on-ground implementation, translating the vision of digital-first care into practical systems change.

To build awareness and capacity across sectors, WJCF facilitated 41 national and state-level conferences across 11 states, engaging over 5,000 participants through ABDM information kiosks and booths. More than 20 webinars and workshops were conducted for over 2,000 healthcare professionals, introducing ABDM workflows and privacy safeguards while promoting the benefits of digital integration.

These efforts align closely with ABDM's mission to develop a citizen-centric, universally accessible digital health ecosystem. By advancing equity (bridging gaps in patient care), strengthening sustainability (building local ownership of digital health), and supporting innovation (engaging private-sector partners), WJCF's work reinforces ABDM's core principles of accessibility, transparency, and innovation.

> "WJCF has been a strong and dependable partner throughout Uttar Pradesh's ABDM journey. Their support in implementing eight Microsites helped us demonstrate how ABDM adoption can be accelerated among healthcare professionals in small and medium private facilities. From enabling ABHA creation to ensuring health record linkages, their team worked closely with ours to translate strategy into action."
>
> **DR. MOHIT SINGH, JOINT DIRECTOR (ABDM),** GOVERNMENT OF UTTAR PRADESH

Bridging the Digital Gap in Rural Uttar Pradesh: The Jogipur Shift

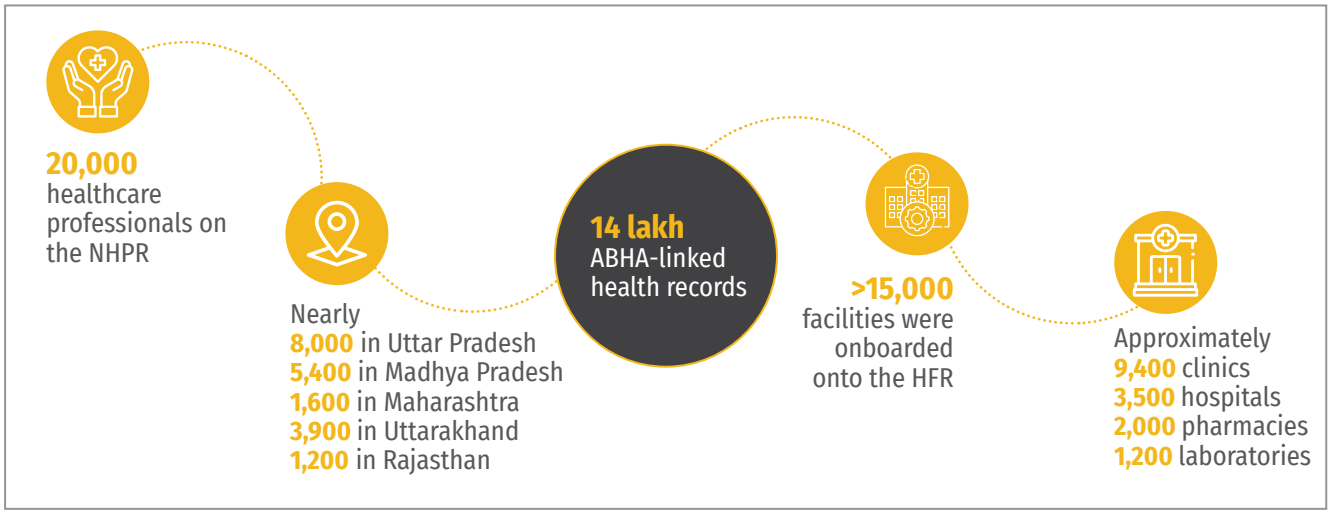
In early 2024, Seth Naval Kishore Urmila Devi Chikitsalaya—a rural hospital in Hardoi, Uttar Pradesh—began its ABDM digitization journey. By April 2024, the facility had a new Hospital Management Information System (HMIS) in place, but it faced a major hurdle: most of its patients, primarily from nearby Jogipur village (population ~3,000), were reluctant to share their Aadhaar IDs or OTPs required for creating ABHA digital health IDs. As a result, only about 150 ABHA-linked records were generated by June 2024, and the facility eventually stopped creating new ABHA records altogether.

PHOTO: WJCF

FIGURE 17. ABHA awareness and creation camp being organized in Jogipur.

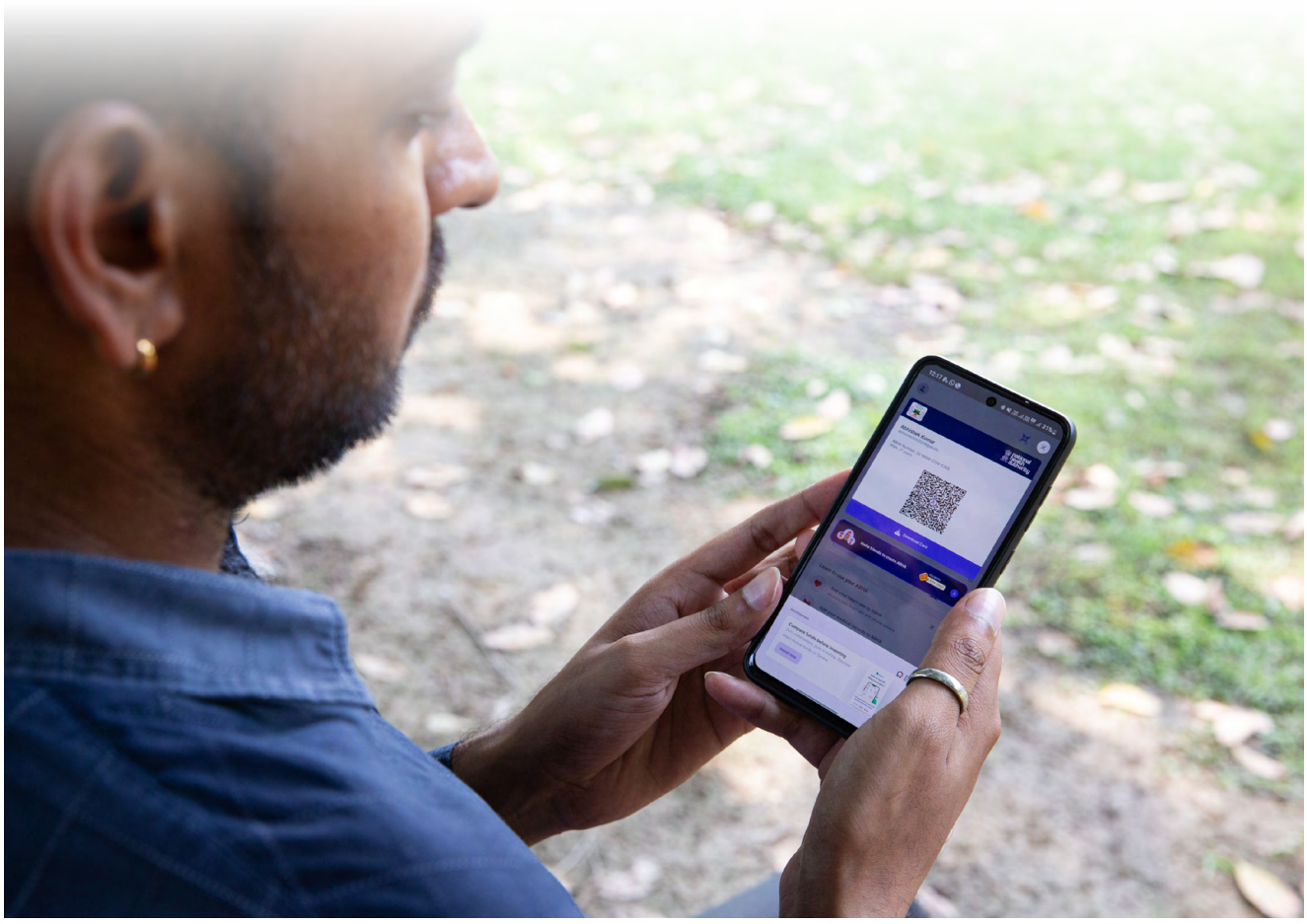
Recognizing the need to build community trust and awareness, WJCF organized a four-day ABDM awareness and ABHA creation camp in Jogipur in July 2024, mobilizing local ASHAs and village leaders (Figure 17). With support from the village pradhan (headman) and ASHA workers, the camp created over 600 new ABHA IDs and educated residents on the benefits of digital health records. Following this intervention, the health facility's ABHA-linked records jumped from 159 to over 1,000 in just four months, and continued to climb to 4,600 by May 2025. The model was endorsed by the district Chief Medical Officer (CMO) and has been replicated in neighbouring villages, demonstrating how community engagement can unlock digital health adoption at the grassroots.

Field information captured during the ABHA awareness and creation camp on 15 July 2024 at Jogipur village, Shahabad block, Hardoi district, Uttar Pradesh; interviewees – Mr. Munner (Village Pradhan) and Mrs. Amita (ASHA).



Looking Ahead

WJCF aims to scale the learnings from high-performing Microsites to the recently initiated Microsites in Rajasthan, while also supporting the NHA in institutionalizing engagement with medical associations and forging new public–private partnerships. Some Microsites will serve as innovation testbeds to pilot interventions that clearly demonstrate ABDM's value, and WJCF will employ demand-side strategies (community outreach, patient education) to drive sustained adoption of digital health tools at health facilities and in communities. Additionally, WJCF plans to conduct a gender-focused study to examine how gender dynamics influence the awareness and uptake of digital health services (including ABDM) among healthcare seekers. These efforts will be underpinned by continued policy engagement and development of tools to strengthen digital health ecosystems at both the central and state levels.



Strengthening Patient Navigation through AB Niramayam Helpline

Target States/Districts:
Madhya Pradesh (Bhopal and Sehore districts)

WJCF Role:
Strategic Advisor, Implementation Support

Donor:
Gates Foundation

Partners & Collaborators:
State Health Agency, Madhya Pradesh (SHA-MP)

The Government of Madhya Pradesh has made notable progress towards universal health coverage by improving access to equitable, affordable care through initiatives like ABDM, Ayushman Aarogya Mandirs, and Pradhan Mantri Jan Aarogya Yojana (PM-JAY). However, patients still face barriers to timely and appropriate care due to lack of information on service availability, difficulty navigating the system, and uncertainty about service delivery, especially in resource-constrained settings. These gaps can lead to delayed care seeking, higher out-of-pocket expenditure (OOPE), and a fragmented and stressful patient journey.

To address these challenges, WJCF partnered with the state health agency in Madhya Pradesh to pilot a call centre–based solution via the existing Ayushman Bharat Niramayam helpline (1800-233-2085). Launched in April 2024 across public health facilities in two districts, the pilot was expanded in March 2025 to include PM-JAY-empanelled private hospitals (Figure 18).

Trained call centre executives now help callers book outpatient consultation and diagnostic appointments across public and private hospitals. Instead of waiting in long lines to register and then again to see the doctor, patients can book an appointment from home and receive priority when they arrive for their slot. Using an AI-powered symptom checker, agents guide patients to the appropriate specialists based on their health needs and proximity. They also help create ABHAs and link them to electronic health records (EHRs), improving the continuity of care under ABDM.

In FY 2024–25, the helpline facilitated over 24,500 appointments across 132 public and 112 private PM-JAY-empanelled hospitals. The network now spans 244 hospitals and 1,466 healthcare providers across 33 specialties, offering 30 diagnostic services and more than 21,000 weekly appointment slots. During the year, 20,232 unique patients used the helpline, which handled 21,500 calls, generating nearly 29,400 service requests, often covering multiple needs, such as consultations and tests.

Performance outcomes point to strong service reliability: 99% of booked appointments were completed, and 83% of patients were seen within 30 minutes of their scheduled time. 14% have become repeat users, indicating growing satisfaction and confidence in the service.

To ensure smooth implementation and expand awareness, over 1,000 hospital staff were trained to manage appointments and on-site coordination, and nearly 4,800 frontline workers, including ASHAs and ANMs, were oriented to promote the helpline within communities (Figure 19). A statewide communications campaign reinforced these efforts through radio messages, WhatsApp and Facebook outreach, and local mobilization by ASHAs and self-help groups.

FIGURE 18. An AB Niramayam telecaller booking an appointment.

FIGURE 19. An ASHA worker informing a family about the AB Nirmayan helpline during a house visit.

From Queues to Care: Kavita's Journey with the Niramayam Helpline

Kavita, a 70-year-old resident of Bhopal and survivor of the 1984 gas tragedy, lives with chronic nerve pain that requires regular medical attention. For years, she spent over INR 3,000 monthly on private neurologist visits—an unsustainable cost for her family. In 2024, she began receiving subsidized care at Bhopal Memorial Hospital & Research Centre but still faced long waits, often having to arrive before dawn to secure a consultation token.

Her experience changed when a hospital staff member trained by WJCF under the helpline initiative helped her book appointments through the AB Niramayam helpline. Since then, Kavita has avoided early-morning queues and secured confirmed time slots while reducing her monthly health expenses by nearly 70%. She now receives consistent, affordable care and feels empowered to manage her condition independently.

Field information captured on 29 May 2025 at BMHRC hospital, Huzur, Bhopal, Madhya Pradesh; beneficiary name changed to protect identity.

> "When I book an appointment on behalf of children in my village, I am assured that the specialist (paediatrician) will be available at the civil hospital. I've used the service about 30 times in the last year."

HEMLATA MALOTIYA, ASHA WORKER,
ASHTA BLOCK, SEHORE DISTRICT, MP

Looking Ahead

In FY 2025–26, the helpline support will expand to three additional districts of Madhya Pradesh with an aim of refining the model for statewide scale-up, and full program ownership would be transferred to the SHA-MP. This next phase will prioritize sustainability, user experience, and digital integration.

Strengthening Medical Oxygen Systems for Resilient Public Healthcare

Target States/Districts:
Chhattisgarh (33 districts), Madhya Pradesh (51 districts), Punjab (23 districts)

WJCF Role:
Technical Assistance and embedded Program Management Unit – Medical Oxygen (PMU-MO)

Donor:
Clinton Health Access Initiative/Gates Foundation

Partners & Collaborators:
Departments of Health and Family Welfare – Chhattisgarh, Madhya Pradesh, Punjab

Hypoxemia, or low blood oxygen, is a serious but often overlooked complication of many conditions including pneumonia, preterm birth, surgical procedures, childbirth, and chronic respiratory illnesses. The COVID-19 pandemic sharply highlighted the importance of medical oxygen, but hypoxemia remains a major cause of preventable mortality beyond the pandemic.

In response to COVID-19, the Government of India and state governments invested substantially in strengthening the country's oxygen infrastructure. Sustaining these gains requires trained staff, routine maintenance, reliable data, and adequate financing. To ensure lasting impact, the state governments of Chhattisgarh, Madhya Pradesh, and Punjab, in partnership with WJCF, are actively integrating oxygen systems into routine health services and expanding hypoxemia management across all levels of care.

At the governance level, WJCF worked with state health departments to develop oxygen guidelines, SOPs, and accountability frameworks while supporting the creation of state-level Oxygen Technical Committees. Regular monitoring of patients' oxygen saturation (SpO_2) levels has now been institutionalized, and over INR 20 crore has been earmarked in annual state budgets specifically for oxygen systems. At the national level, WJCF played a key role in supporting the MoHFW's release of updated guidelines on medical oxygen management.

Across the three states, WJCF's support spans all district hospitals, all sub-district and civil hospitals (Chhattisgarh: 14, MP: 89, Punjab: 41), and over 630 community health centres (Chhattisgarh: 169, MP: 306, Punjab: 161). Focused support is being provided for pilot initiatives across 5% of primary health and wellness centres to strengthen the last-mile availability of oxygen (Figure 20).

To improve supply chain efficiency and infrastructure readiness, WJCF helped establish supportive supervision systems and designed checklists to assess the

FIGURE 20. A nurse at District Hospital Raisen, Madhya Pradesh records SpO_2 levels during a routine patient check, a practice now integrated into standard inpatient care.

functionality of oxygen equipment. Nationwide mock drills were conducted, covering 1,200+ pressure swing adsorption (PSA) oxygen generation plants, with WJCF providing real-time data analysis and field support. Internal dashboards were created to track key supply indicators, while costing tools were introduced to help states optimize the procurement and production of medical oxygen. Facilities with limited access to oxygen concentrators were identified, enabling the state government to carry out targeted redistribution to over 200 primary care facilities and further improved frontline access.

Capacity-building efforts included a first-of-its-kind Training of Trainers (ToT) program, through which over 1,000 healthcare workers were trained in oxygen system management, safety, and troubleshooting (Figure 21). Oxygen management modules have now been integrated into state training programs to sustain learning. A comprehensive toolkit including checklists, 'Dos and Don'ts', and best practices was institutionalized across facilities to standardize service delivery.

At the national level, WJCF also provides technical assistance to MoHFW through the Program Management Unit for Medical Oxygen (PMU-MO), responsible for planning new booster compressor sites, coordinating technical trainings, and engaging with states to improve oxygen ecosystem functionality. Together, these efforts are ensuring that oxygen availability and use are embedded within India's routine health systems, making facilities better prepared, health workers more confident, and care delivery more resilient.

> "Medical oxygen is not just a resource; it's a lifeline. With WJCF's support, we are strengthening our medical oxygen ecosystem. Their technical expertise, hands-on capacity building, and consistent systems support have empowered our teams on the ground to respond more effectively and save lives."

DR. D.K. TURRE, DEPUTY DIRECTOR (HOSPITAL ADMINISTRATION), DIRECTORATE OF HEALTH SERVICES, CHHATTISGARH; AND **DR. ANIL SHUKLA, DIVISIONAL JOINT DIRECTOR,** HEALTH SERVICES, SURGUJA, CHHATTISGARH

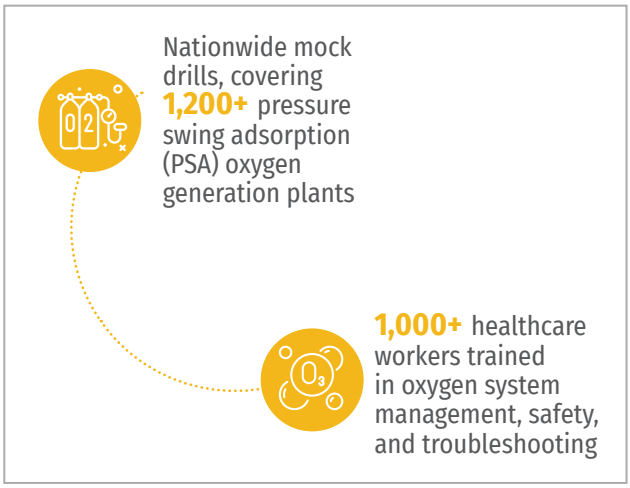


FIGURE 21. Healthcare workers in Civil Hospital Sanchi, Raisen district, Madhya Pradesh, receive hands-on training on the proper use and handling of oxygen equipment.

Strengthening Neonatal and Paediatric Care through Oxygen Systems

In Chhattisgarh's Raipur district, a premature baby girl, Rashi, was admitted to the NICU at the District Hospital, Raipur, in 2024 with multiple complications, including low birth weight, jaundice, and sepsis. She received oxygen therapy and clinical care over an 18-day stay and was discharged in stable condition (Figure 22). However, just five months later, the same child returned with acute bronchiolitis and hypoxemia (SpO_2 89%). She was promptly stabilized with oxygen therapy and antibiotics and discharged after eight days with home care guidance.

This timely response, on both occasions, was possible due to a set of systems now firmly in place. With support from WJCF, the state has integrated SpO_2 monitoring as the fifth vital sign in inpatient case sheets—a change that ensures oxygen levels are checked for every admitted individual. The healthcare workers had also received regular training on oxygen therapy. These combined efforts meant the child's hypoxemia was recognized early, and care could begin without delay.

FIGURE 22. Neonates under continuous SpO_2 monitoring in a Special Newborn Care Unit (SNCU); image for representation only.

Field information captured during an interview on 7 June 2024 at Raipur district, Chhattisgarh; beneficiary name changed to protect identity.

Looking Ahead

WJCF will continue supporting the integration of oxygen systems into routine public health services. Upcoming priorities include helping states develop comprehensive oxygen roadmaps with clear actions, responsibilities, and budgets for sustainable operations and maintenance; creating forecasting tools that link oxygen demand to disease trends and hospital data; digitizing training modules and integrating them into government e-learning platforms; developing IEC materials and KAP (knowledge, attitude, practice) tools to institutionalize best practices in oxygen use; and strengthening data and governance systems so that oxygen indicators are embedded in routine health program reviews.

Climate and Health

This image is for representation purpose only. Source: freepik.com.

Transforming India's Air Conditioning Market to Achieve Energy Efficiency

Target States/Districts:
Pan-India (with a pilot in Delhi)

WJCF Role:
Market Coordinator

Donor:
Clinton Health Access Initiative

Partners & Collaborators:
Bureau of Energy Efficiency (BEE), Ministry of Power; Voltas; Daikin; Godrej; AHEAD Program (DPIIT–World Bank); International Finance Corporation (IFC); Tata Power–DDL; Croma; Attero (recycler)

India's demand for cooling is projected to rise sharply—over 100 crore air conditioners could be installed by 2050. Yet currently, less than 10% of households own an AC. Many first-time buyers opt for less efficient models due to higher upfront costs of high-efficiency models (a 5-star AC can cost ~INR 6,800 more than a 3-star unit), while some two crore old, inefficient ACs remain in use beyond their recommended lifespan. These factors strain electricity grids and drive up carbon emissions.

WJCF is catalyzing a transition to high-efficiency cooling in India, supporting government objectives under the India Cooling Action Plan (ICAP) and Montreal Protocol commitments. We are addressing both demand-side affordability and supply-side readiness, coordinating diverse stakeholders to develop scalable market models. Through a market-transformation approach, WJCF collaborated with a coalition of key partners, including four major AC manufacturers (~40% market share), a leading retail chain, India's primary AC recycler, development finance institutions (such as the World Bank), and government agencies, like the Bureau of Energy Efficiency (BEE). Partnerships also extend to civil society networks through the UN Cool Coalition and India Cooling Coalition.

Early engagement with manufacturers revealed that the Indian AC industry faces constraints in scaling next-generation efficient technologies due to heavy import dependence for key components and technology. In response, WJCF expanded its role to address these systemic challenges. We provided technical inputs to the Government of India and the World Bank's $700 million AHEAD cooling program, ensuring that new domestic manufacturing investments adopt an efficiency-first approach. WJCF also supported the inclusion of efficient AC manufacturing in Quad Alliance commitments, laying the foundation for global collaboration.

FIGURE 23. A promotional standee displayed at a TPDDL customer centre to inform visitors about the energy rebate available on energy-efficient 5-star ACs for first-time buyers.

To overcome affordability barriers for first-time AC buyers, WJCF partnered with Tata Power Delhi Distribution Ltd (TPDDL), a leading utility in Delhi, to study India's first utility-led energy rebate program. Under this study, the first of its kind in India, TPDDL offered an upfront rebate to customers purchasing a high-efficiency 5-star AC, making its price comparable to a lower-rated model. Retail partners and financiers were also engaged to simplify the customer experience. Utilities proved to be a promising anchor for such incentive models, given the grid-level benefits of energy-efficient appliances (Figure 23).

The study's operational success enabled TPDDL to submit a proposal to the Delhi Electricity Regulatory Commission to independently scale the model to 10,000 beneficiaries

over the next three years. Insights from this study are shaping future consumer incentive programs. TPDDL has also brought these lessons into policy discussions with the Ministry of Power on broader energy-efficiency initiatives.

> “Through this partnership, we learned that rebates are far more effective when delivered in collaboration with retailers, where purchase decisions are actually made—and that combining rebates with consumer financing significantly improves affordability. These insights are now foundational to how we design future energy-efficiency programs.”
>
> **SYED HASAN, HEAD OF ENERGY-AS-A-SERVICE,** TPDDL

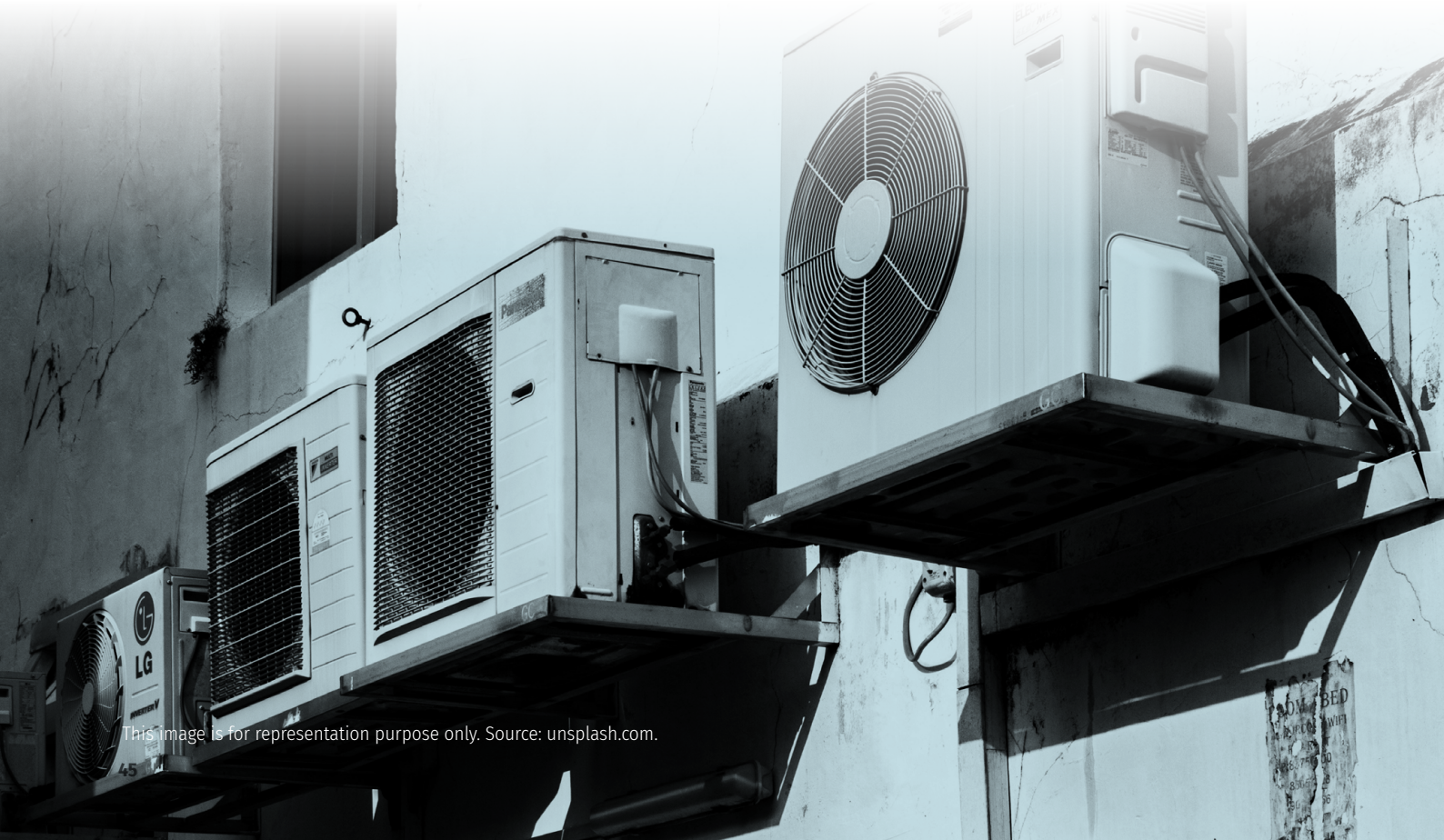
In parallel, WJCF co-developed a blended finance mechanism with a leading development finance institution that combines concessional capital, manufacturer rebates, and retail incentives into a single consumer offer. This integrated approach is expected to double India’s AC replacement rate, unlocking hundreds of millions of dollars in financing and enabling the replacement of approximately 3,00,000 old, inefficient ACs with high-efficiency models.

WJCF’s technical guidance has also informed BEE’s development of India’s first national AC replacement policy, which aims to accelerate the replacement of inefficient models with high-efficiency ones. WJCF recommended retailer- and financier-driven implementation models to enhance customer reach and streamline delivery. This policy push, coupled with our ongoing engagement with major manufacturers, retailers, and financiers, has strengthened alignment between industrial, financial, and consumer priorities in India’s cooling transition.

Looking Ahead

To accelerate the replacement of outdated ACs, WJCF will continue supporting government agencies such as BEE in designing and implementing national replacement policies. We will also launch a consumer finance credit line with a leading development finance institution and mobilize manufacturers, retailers, and financiers to offer additional consumer discounts. This coordinated model will make 5-star ACs more affordable while systematically phasing out inefficient units.

In parallel, WJCF will work with the Ministry of Power to shape a national rebate program for first-time buyers, drawing on insights from the TPDDL pilot. Partnering with a major retail chain, we will pilot innovative consumer outreach and financing solutions to expand access to energy-efficient cooling, reducing household energy costs and easing the strain on the power grid.



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Non-Communicable Diseases

Strengthening Access to Type 1 Diabetes Care

Target States/Districts:
Chhattisgarh (Durg, Kanker, Raipur districts), Madhya Pradesh (Bhopal Division and Jabalpur divisions – 16 districts), Rajasthan (all 42 districts), Uttarakhand (Dehradun, Haridwar, Rudraprayag, Tehri districts)

WJCF Role:
Strategic Advisor, Implementation Support, Technical Partner

Donor:
Breakthrough T1D

Partners & Collaborators:

Government Partners: Ministry of Health & Family Welfare (MoHFW); technical expert group on NCDs in children; National Health Mission/Departments of Health – Chhattisgarh, Madhya Pradesh, Rajasthan, Uttarakhand

Clinical & Research Institutions: AIIMS Bhopal, AIIMS Jodhpur; AIIMS Rishikesh; Maulana Azad Medical College (Delhi); Lok Nayak Hospital (Delhi); Aruna Asaf Ali Government Hospital (Delhi)

Community: Type 1 Diabetes peer-support groups and community networks

Type 1 Diabetes Mellitus (T1D) is an autoimmune condition in which the body produces little or no insulin. Without reliable access to basal–bolus insulin, self-monitoring of blood glucose 3–4 times a day, age-appropriate diabetes education and counselling, and regular follow-up, T1D imposes significant health, social, and financial burdens. However, having these fundamentals in place can enable people living with T1D (PLT1D) to lead long, healthy, and productive lives.

India accounts for the world's second-highest share with an estimated 8,20,000 PLT1D, including ~2,76,000 children and adolescents. In India, the estimated life expectancy of a child diagnosed with T1D at age 10 is only 40 years—significantly lower than in high-income countries, where life expectancy is comparable to that of the general population.

The Government of India has established a strong framework for non-communicable disease (NCD) care through the National Programme for Prevention and Control of NCDs (NP-NCD). This provides a robust platform to expand and integrate care for T1D, including for children and adolescents. WJCF is supporting MoHFW and state health departments to strengthen this ecosystem with the goal of enabling PLT1Ds to live longer, healthier, and more productive lives.

WJCF has partnered with the state health departments of Chhattisgarh, Madhya Pradesh, Rajasthan, and Uttarakhand to integrate T1D care into existing NCD clinics in government hospitals. This has involved developing and adopting standardized care guidelines, building healthcare provider capacity, ensuring facility readiness and supply of insulin and glucose-monitoring commodities (Figure 24), operationalizing weekly T1D clinics, and strengthening monitoring, referral, and governance structures to drive service uptake and inform decision-making.

FIGURE 24. A PLT1D in Rajasthan with his monthly supply of blood sugar monitoring supplies.

FIGURE 25. Healthcare workers being trained on T1D symptoms and counselling support.

Between 1 April 2024 and 30 April 2025, significant progress was recorded, with 900 PLT1Ds (52% female, 48% male) enrolled and receiving regular care at the T1D clinics, and with 95% now receiving a standardized basal–bolus insulin regimen—up from 35% at baseline. Over 70 physicians, paediatricians, and medical officers were trained on T1D diagnosis, initiation of treatment, and management of care. Additionally, over 1,100 staff nurses, ASHAs, and RBSK medical officers were oriented on T1D signs and symptoms to enable timely detection, facilitate enrolment, and extend education and counselling support (Figure 25). These initiatives are enabling detection, improving access to standardized basic care, and supporting self-management of care through access to essential drugs and commodities and age-appropriate education.

WJCF has supported its partner states in adopting standardized T1D care guidelines to ensure uniform service delivery. Rajasthan has further institutionalized these efforts through its Mission Madhuhaari initiative, scaling T1D services across all the districts in the state (Figure 26). Complementary awareness and education campaigns are building provider and caregiver understanding of T1D, empowering families to better manage the condition.

Experience and learnings from pilot implementation T1D services across multiple states are informing the need for national guidelines to help incorporate T1D care within the ambit of existing Non-Communicable Diseases (NCD) and child health programs. This will ensure access to basic T1D care through the NCD clinics at district/sub-district hospitals and community health centres. Through this comprehensive approach, WJCF is helping embed T1D care within India's broader NCD and child health ecosystem—strengthening systems, improving treatment quality, and enabling children, adolescents, and adults with T1D to lead healthier lives.

> "Type 1 Diabetes program has been a valuable initiative for strengthening our approach to managing T1D at the facility level. The training was comprehensive and practical, enhancing our capacity for timely diagnosis, insulin therapy, and patient follow-up. It holds strong potential to improve long-term outcomes for individuals living with T1D in our communities."
>
> **DR. GARVITA HAJELA, MEDICAL OFFICER,**
> NCD CLINIC, SDH MELA, HARIDWAR

PHOTO: RAJASTHAN GOVERNMENT

FIGURE 26. The Chief Minister of Rajasthan announcing a statewide scale-up of Mission Madhuhaari.

Care Close to Home: How a Local T1D Clinic Is Changing Lives

Narendra, a 10-year-old boy from a small town near Jaipur, Rajasthan, was diagnosed with Type 1 Diabetes in April 2024. Due to the lack of T1D care at the district hospital, his family had to travel to Jaipur city every month for consultations and essential supplies. Each visit cost about INR 1,600–1,700, took over four hours, and resulted in lost wages for the family and missed school for Narendra. Limited access to insulin and blood glucose–monitoring supplies added financial strain and made consistent care difficult.

This changed with the launch of a dedicated T1D clinic at the Sub-District Hospital (SDH) Shahpura in December 2024. The clinic was set up under Mission Madhuhaari, a statewide initiative jointly conceptualized by the Government of Rajasthan and WJCF to strengthen access to standardized T1D care. With the adoption of standard T1D care guidelines, training of service providers/clinicians, and regular provision of insulin and glucose-monitoring supplies, SDH Shahpura began offering free T1D services at its NCD clinic.

Now, Narendra and his family can access the care they need just 15 minutes from home. The availability of essential drugs and supplies for daily T1D management has significantly reduced the family's financial burden and travel time, enabling them to return to work and school. T1D has transformed from a monthly crisis into a manageable condition for Narendra's family.

Field information captured on 21 April 2025 at Shahpura, Rajasthan; interviewee – Narendra, a person living with T1D (beneficiary).

Looking Ahead

Beyond the four pilot states, WJCF aims to support the integration of T1D services in additional states by adapting lessons learned from the pilots. At the national level, the team is working closely with leading T1D peer-support groups to co-develop strategies enabling peer support for those enrolled in T1D clinics. WJCF is also collaborating with public medical institutions in Delhi to pilot an early screening tool for T1D, an important step toward timely diagnosis and intervention for at-risk children. Together, these efforts are shaping a scalable, sustainable, and person-centred ecosystem for T1D care in India.

Strengthening Access to Screening and Pre-Cancer Treatment of Cervical Cancer

Target State/District:
Rajasthan (all districts)

WJCF Role:
Strategic Advisor, Implementation Support, Technical Partner

Donors:
Clinton Health Access Initiative/UNITAID

Partners & Collaborators:
NCD Department (Department of Medical, Health & Family Welfare), Government of Rajasthan

Cervical cancer remains the second leading cause of cancer among Indian women, despite being largely preventable. It accounts for an estimated 1,28,000 new cases and 80,000 deaths in India annually. Because cervical cancer has a long pre-cancerous stage, early detection and access to treatment for pre-cancerous lesions can dramatically reduce mortality. Yet, according to NFHS-5, less than 2% of women in India have ever been screened for cervical cancer. Barriers to care include low awareness and knowledge, limited geographic and financial access, gender-related challenges, and privacy concerns.

In 2019, WJCF—with funding support from the Clinton Health Access Initiative (CHAI) and Unitaid—partnered with the Government of Madhya Pradesh to strengthen cervical cancer prevention services. Starting as a pilot across 15 districts, WJCF helped Madhya Pradesh scale screening and treatment services to all 55 districts, while also providing support to the state governments of Chhattisgarh, Punjab, Sikkim, Uttar Pradesh, and Rajasthan.

In 2022, with CSR support from Birlasoft, WJCF extended its assistance to Rajasthan to strengthen and decentralize cervical cancer prevention services across tertiary- and secondary-level facilities in the state.

Across 68 public health facilities, comprising government medical colleges, district and sub-district hospitals, WJCF has supported the availability of commodities for cervical cancer screening through Visual Inspection with Acetic Acid (VIA), capacitated gynaecologists, medical officers, and staff nurses to strengthen screening services, and deployed 63 portable thermal ablation devices to enable a screen-and-treat approach to prevent cervical cancer (Figure 27). To ensure quality service delivery, 123 healthcare providers, including gynaecologists, medical officers, and nurses, have been trained to conduct VIA screening and use thermal ablation (TA) for cervical

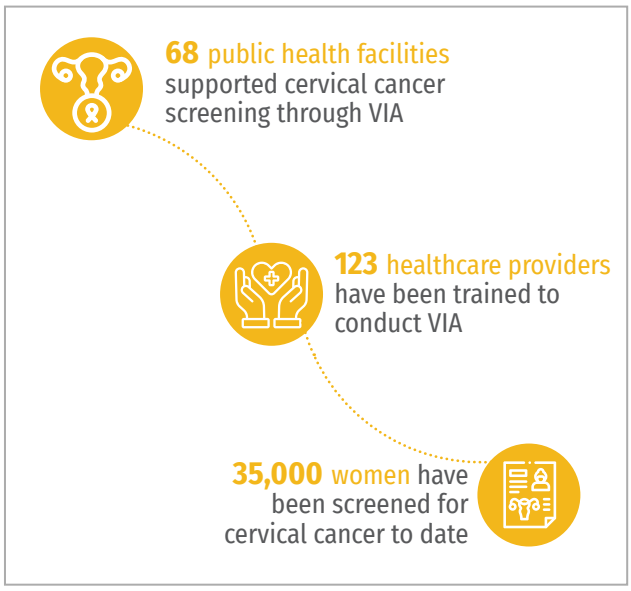
FIGURE 27. Commodities for pre-cancerous screening, and a thermal ablation device.

FIGURE 28. Training of gynaecologists on the cervical cancer screening and pre-cancer treatment using portable TA devices.

pre-cancer treatment (Figure 28). Facility assessment and process optimization have strengthened referral linkages and reporting systems, improving follow-up and continuity of care.

As a result, over 35,000 women have been screened for cervical cancer to date, with those screened positive having access to timely pre-cancer treatment. These outcomes reflect significant progress in expanding access to screening and ensuring prompt treatment for women at risk.

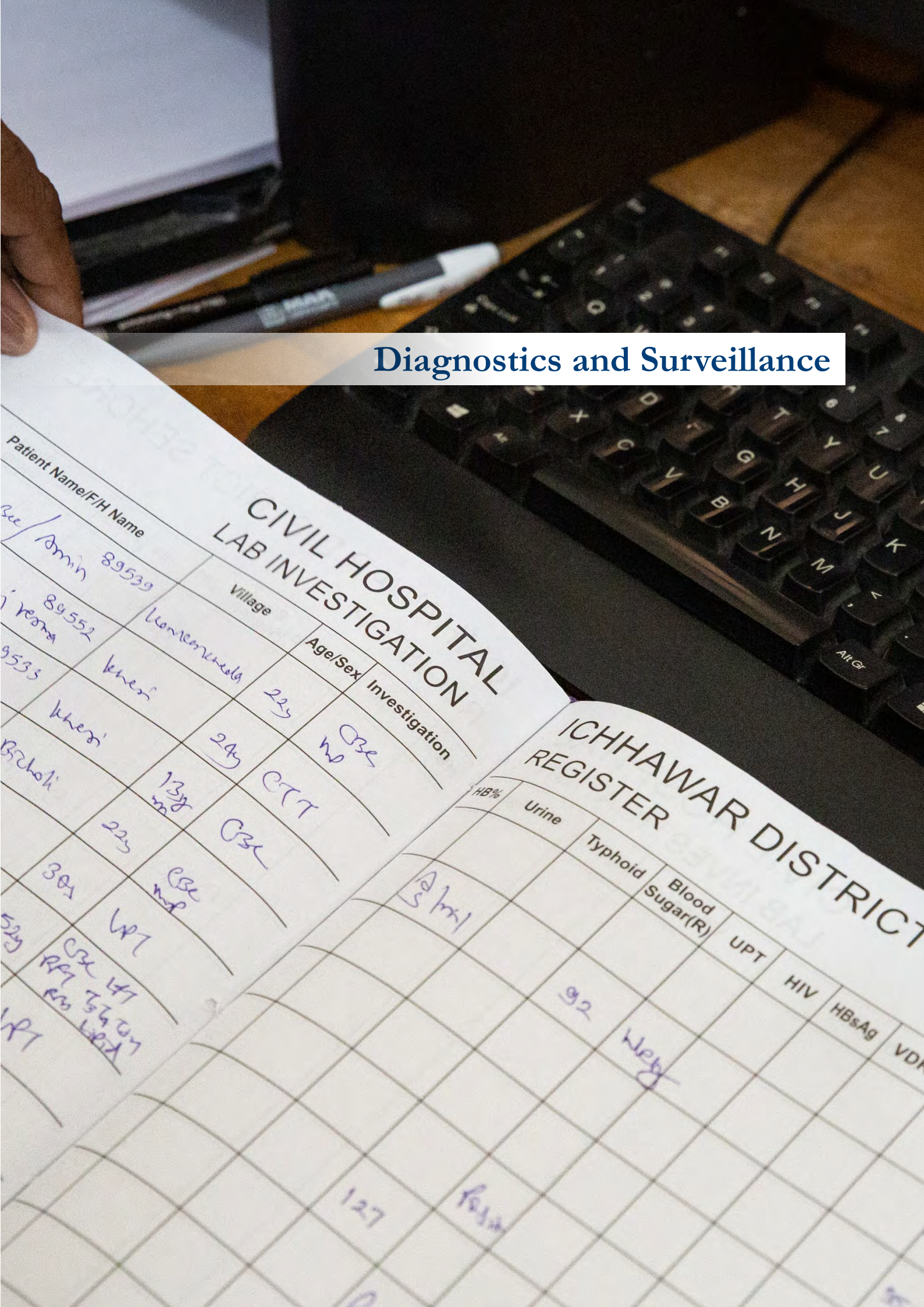
To drive awareness and community engagement, WJCF supported the development and dissemination of impactful IEC materials and capacitated ASHA workers in mobilizing eligible women for screening. In collaboration with the Government of Rajasthan, WJCF supported the adoption of the 'Fit Health Worker' campaign, encouraging and enabling screening for NCDs, including cervical cancer, for healthcare providers and frontline community health workers. This, while extending the benefits of timely screening, helped create 'informed champions' equipped with the knowledge and understanding on the benefits of early screening to advocate for and mobilize eligible beneficiaries for cervical cancer screening from their respective communities.



> "I learned about cervical cancer screening and pre-cancer treatment during the training workshop. I even got myself screened during a camp at the district hospital. Now I encourage all eligible women in my community to get screened."
>
> **CHANDA KUMARI, ASHA WORKER,**
> JAIPUR, RAJASTHAN

Looking Ahead

Building on the positive impact of this program, WJCF is engaging with the state government of Rajasthan to collaborate on further decentralizing cervical cancer prevention services down to the community and primary care levels. The next phase will significantly expand screening coverage at the grassroots. In addition to scaling up services, this initiative will inform national policy discussions by sharing successes, learnings, and evidence from the decentralization of cervical cancer prevention services. It will also pilot the integration of cervical cancer prevention services for women living with HIV (WLHIV), who are at a sixfold greater risk of cervical cancer, by integrating screening and treatment services at antiretroviral therapy (ART) centres. Moreover, the program will enable better access to pre-cancer treatment through portable thermal ablation devices, bridging critical gaps and helping institutionalize preventive care across all levels of the health system.


Diagnostics and Surveillance
CIVIL HOSPITAL
LAB INVESTIGATION
Patient Name/F/H Name
Village
Age/Sex
Investigation
ICHHAWAR DISTRICT
REGISTER
HB%
Urine
Typhoid
Blood Sugar(R)
UPT
HIV
HBsAg

Strengthening Public Health Surveillance and Diagnostics

Target States/Districts:
Madhya Pradesh (20 focus districts: Dhar, Ujjain, Chhatarpur, Vidisha, Satna, Mandsaur, Bhind, Bhopal, Jhabhua, Betul, Gwalior, Indore, Singrauli, Katni, Alirajpur, Ashoknagar, Damoh, Khandwa [East Nimar], Harda, Dindori) and Pan-India

WJCF Role:
Strategic Advisor, Implementation Support, Technical Partner, Policy Engagement

Donor:
Clinton Health Access Initiative/Gates Foundation

Partners & Collaborators:

Government of India: Central Surveillance Unit; National Centre for Disease Control (NCDC)

Government of Madhya Pradesh: State Surveillance Unit; Integrated Disease Surveillance Program (IDSP)

Technical Partners: World Health Organization (WHO); JHPIEGO (technical support in training and implementation)

The COVID-19 pandemic highlighted gaps in India's real-time disease surveillance, including the slow uptake of modern digital surveillance systems, under-reporting from the private sector, delayed outbreak detection, and uncoordinated response mechanisms. These challenges underlined the need for an integrated, responsive, digitally enabled surveillance system capable of withstanding future health crises. In this context, the Public Health Surveillance (PHS) project was conceptualized under the leadership of the Central Surveillance Unit of the National Centre for Disease Control (NCDC), working with state surveillance units and key partners, like the Gates Foundation and WJCF.

The PHS project aimed to complement national priorities by enhancing the Integrated Health Information Platform (IHIP), improving its digital infrastructure, data reporting and use, and by building human resource capacity, thereby moving from reactive crisis management to proactive public health preparedness.

WJCF's support was anchored on four pillars: digital transformation, enhanced governance, workforce development, and data integration (including private-sector data). All efforts were designed to be embedded within public systems for sustainability.

WJCF's support to strengthen disease surveillance in Madhya Pradesh was anchored on four pillars: digital transformation, enhanced governance, workforce development, and data integration (including private-sector data). All interventions were designed for sustainability by embedding them within government systems.

In 2024, this vision took shape through two flagship initiatives with the Government of Madhya Pradesh. The Early Warning, Alert and Response System (EWARS), a real-time, threshold-based alert mechanism co-created with the state government, was piloted across 20 districts covering approximately 3.4 crore people. The system detects and responds to outbreaks of more than 26 priority diseases using data from the IHIP. During the pilot, 342 EWARS alerts were generated, of which 26 were confirmed as outbreaks through field investigations under the Integrated Disease Surveillance Programme (IDSP). All alerts received a timely response from district surveillance units, with a 100% response rate recorded. Notably, 65% of alerts generated from laboratory data (L-forms) were confirmed by field verification, compared to only 7% from presumptive diagnosis (P-forms), highlighting the stronger predictive value of lab-based surveillance and reinforcing the importance of data-driven public health decision-making.

Complementing this effort, the Private-Sector Engagement (PSE) initiative was launched to strengthen reporting

> "Increasing private-sector reporting on IHIP was a complex challenge for us, with limited engagement, insufficient incentives, and gaps in the onboarding process. Strategies tested in collaboration with WJCF under the pilot proved especially effective. We are now building on the partnerships forged during the pilot to scale up these initiatives across the entire state."
>
> **DR. SHAILENDRA KUMAR SINGH, STATE ENTOMOLOGIST,** STATE SURVEILLANCE UNIT (IDSP), MADHYA PRADESH

Linking Care to Surveillance

Lupin Diagnostics, a private lab in Bhopal, previously managed its test data on a standalone Laboratory Information Management System (LIMS). Due to limited awareness and workflow constraints, the lab had not been submitting critical disease data to the government's surveillance network.

Through the Private-Sector Engagement pilot, WJCF field staff provided targeted training on IHIP reporting and helped align the lab's data formats with national reporting standards, demonstrating the new reporting process at the facility (Figure 29). As a result, Lupin Diagnostics now reports routinely on the government portal, significantly enhancing disease surveillance in the region. The lab also benefits from weekly bulletins on local disease trends, which help it optimize stock allocation and improve planning for test supplies.

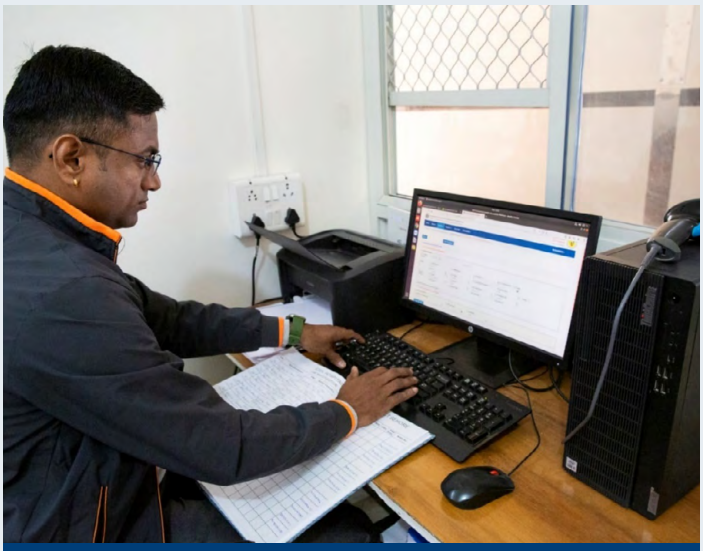


FIGURE 29. Private laboratory staff member successfully reporting data onto government portal for better disease surveillance using developed reporting tools.

Field information captured on 28 December 2023 at Huzur, Bhopal district, Madhya Pradesh.

from non-government health facilities, which account for a significant share of healthcare delivery. WJCF supported the onboarding of 118 private facilities onto IHIP across three districts using streamlined reporting templates, targeted training, and communication materials. As a result, the frequency and volume of disease reporting improved substantially, with a 52% increase in the average number of days per week that patient data was reported, and a 138% rise in laboratory (L-form) reporting. The number of reported cases also grew by 31% for P-form facilities, and 46% for L-form facilities. These improvements reflect stronger data culture and participation among private providers, driven by consistent engagement with professional medical associations, such as the Indian Medical Association and the Indian Academy of Pediatrics. The pilot has since been scaled statewide under government leadership.

To further institutionalize surveillance capacity, WJCF supported the development and dissemination of five thematic disease surveillance dashboards that were first piloted in Madhya Pradesh and later scaled nationally on the IHIP platform. Additionally, more than 5,000 copies of the IDSP Outbreak Response Manual were printed and distributed across the state, with the manual formally launched by the Hon'ble Chief Minister of Madhya Pradesh.

Looking Ahead

WJCF has transitioned much of this work to the central and state surveillance units, with key interventions scaling up under government ownership. WJCF continues to provide technical assistance to IDSP and NCDC while identifying and addressing remaining gaps in public health surveillance. Upcoming priorities include expanding early warning systems by integrating climate and environmental health data (especially for vector-borne diseases), strengthening public–private partnerships for reporting, bolstering community-based surveillance, and adopting One Health approaches to integrate traditionally siloed domains.

Open PCR Market Landscaping to Unlock Affordable Molecular Testing Infrastructure

Target States/Districts:
Pan-India

WJCF Role:
Strategic Advisor, Technical Partner

Donor:
Clinton Health Access Initiative/Gates Foundation

Partners & Collaborators:
State health departments; IPAQT consortium; diagnostic manufacturers

Polymerase Chain Reaction (PCR) testing has been a cornerstone of disease diagnostics for over 15 years (in HIV and TB programs). The COVID-19 pandemic, however, was a turning point; it led to a rapid expansion of RT-PCR infrastructure across India and proved that high-throughput, 'open' PCR platforms could be deployed at scale. This created an opportunity to reimagine the role of these machines in addressing other public health priorities like TB. In contrast, many programs rely on closed RT-PCR systems which, while user-friendly, cost around INR 800–1,200 per test, are often disease-specific, and depend on centralized supply chains.

Open (distributable) PCR kits, which can run on general-purpose PCR machines, offer a more adaptable and cost-effective alternative with broader disease testing capability and more flexible supply chains. However, lower levels of automation, added operational complexity, and regulatory hurdles have limited their use in public health programs to date.

In 2023, WJCF undertook a pan-India landscaping exercise to evaluate the potential for integrating open PCR platforms into routine disease surveillance and diagnostic services, with a focus on TB. The project focused on three key areas:

1. **Capacity Mapping:** Assessed the distribution and capacity of open PCR instruments.
2. **Defining use cases:** Engaged government and key stakeholders to explore use cases, operational requirements, and barriers to adoption for high-volume TB screening. Findings informed a target product profile and guided discussions with WHO, suppliers, and policymakers.
3. **Defining regulatory pathways:** Documented national regulatory pathways for distributable PCR kits, including ongoing validations by Indian Council of Medical Research (ICMR) for TB assays from Indian manufacturers (Huwel, Mylab, Meril, Cosara, Acconlife, and Neodx).

Key Accomplishments

Infrastructure Opportunity: WJCF mapped an estimated 7,900 RT-PCR platforms in India. Some states are already utilizing these machines beyond COVID-19, for tests like hepatitis C (HCV), hepatitis B (HBV), and H1N1. State officials, however, highlighted operational challenges such as sample transport limitations and the need for additional training to effectively repurpose these machines.

Private-Sector Momentum: Through the IPAQT consortium, WJCF found that private laboratories have embraced open PCR for a wider range of diseases—including HCV, HBV, HIV, TB, influenza, and chikungunya—demonstrating the feasibility and benefits of multi-disease testing on open platforms.

Regulatory Alignment: The landscape analysis identified a growing interest among policymakers in reducing molecular testing costs and diversifying suppliers. This includes exploring both enhanced closed PCR solutions (e.g. new indigenous platforms) and open PCR adoption. ICMR is already validating several open PCR-based TB assays, and one such assay (by Huwel) has received a positive recommendation, indicating readiness for broader use.

Deployment Strategy: While closed PCR systems (like GeneXpert) will remain crucial for peripheral settings due to their simplicity and wide installed base, open PCR holds strong potential for district-level labs that have adequate infrastructure and sample transport networks. States with more robust lab capacity are likely to lead in adopting open PCR, whereas lower-capacity states may require investments in training and operational support. The private sector is expected to move faster in integrating open PCR into its service offerings.

Looking Ahead

WJCF continues to support the Central TB Division (CTD) in improving diagnostic capacity using existing and potential tools. It is exploring avenues for deploying approved RT-PCR assays for active case-finding across UP, Bihar, Meghalaya, and Maharashtra. WJCF is also exploring partnerships with other assay developers, such as Agappe (Sree Chitra Tirunal Institute for Medical Sciences & Technology) to simplify process flows, provide guidance on regulatory clearances, and enable private and public sector integration.

PHOTO: WJCF

New Initiatives

Boosting Access to Sickle Cell Care in Tribal Communities

Donor: Clinton Health Access Initiative/Open Philanthropy

Sickle cell disease (SCD) is a chronic, inherited blood disorder that causes red blood cells to become misshapen, impairing oxygen delivery and leading to severe pain, fatigue, and life-threatening complications. India bears the world's third-highest SCD burden, with over 70% of cases concentrated among tribal populations.

Recognizing this critical public health challenge, the Government of India launched the National Sickle Cell Elimination Mission in July 2023, setting an ambitious goal to eliminate SCD as a public health problem by 2027. Madhya Pradesh, home to India's largest tribal population, is pivotal to achieving this goal. The state has expanded screening coverage significantly, reaching over 90 lakh individuals by March 2025. Building on this momentum, efforts are now focused on timely diagnosis, strengthening treatment pathways (including Hydroxyurea use), ensuring reliable supply chains, and supporting consistent follow-up care, particularly in underserved tribal blocks.

WJCF has been supporting the Government of Madhya Pradesh's SCD program since 2021 as a key partner of the State Haemoglobinopathies Mission. To further the state's elimination vision, WJCF is providing technical, analytical, and implementation assistance across 20 tribal districts (89 blocks) under NHM-MP's leadership. Current efforts focus on ensuring uninterrupted access to diagnostics and Hydroxyurea through improved forecasting and supply chain systems, building provider capacity through standardized, evidence-based training, and driving community awareness using culturally tailored IEC tools. Since the program's launch in 2025, WJCF has conducted a baseline assessment across 40 facilities in eight high-burden districts, supported the rollout of statewide screening targets (covering 30 lakh individuals), improved the availability of hydroxyurea (HU) at sub-district facilities, designed and deployed program dashboards, supported commodity quantification, and trained over 800 healthcare workers across 33 districts to strengthen service delivery and continuity of care (Figure 30).

PHOTO: WJCF

FIGURE 30. State-level workshop on sickle cell disease management convened by NHM–Madhya Pradesh with WJCF technical support.

Looking Ahead

WJCF will focus on deepening implementation in priority districts by strengthening facility-level service delivery and expanding outreach to high-risk groups. Efforts will include rolling out caregiver training modules, supporting rational Hydroxyurea use through mentorship, and integrating SCD services into routine health platforms. Dashboards and review tools will be refined for real-time decision-making at district and block levels. WJCF will also document emerging best practices and work closely with NHM-MP to embed these into policy and operational frameworks, laying the foundation for scalable, sustainable SCD care across the state.

Community-Based Household Contact Management of TB

Donor: Clinton Health Access Initiative/GiveWell

India accounts for nearly one-quarter of the global tuberculosis (TB) burden, so preventive strategies are critical to reducing transmission and meeting national elimination targets. Yet household contacts of TB patients, especially children under five, are often missed in routine TB prevention efforts, despite being at the highest risk of infection.

To address this gap, WJCF, under the leadership of the CTD, has launched a new initiative to strengthen community-based household contact management (CbHCM) for TB. This model emphasizes timely identification, screening, and preventive treatment of TB patients' household contacts (with a focus on under-fives), with the aim of reducing missed opportunities for prevention. The approach integrates community health workers, digital tracking platforms, and decentralized care delivery to improve the coverage and adherence of TB preventive therapy (TPT).

The initiative began with a pilot phase in early 2025 across three high-burden districts in Uttar Pradesh and Bihar. This phase focused on refining key operational workflows, field-testing digital tools, and gathering early insights on system efficiency and patient engagement. These learnings are now informing a full-scale rollout, including embedded implementation research across 70 additional districts in FY 2025–26. The scale-up will be accompanied by robust monitoring metrics to ascertain the model's cost-effectiveness, and a structured policy engagement strategy to share evidence and advocate for broader adoption.

By generating actionable evidence and demonstrating a scalable, patient-centred prevention model, the program aims to contribute to India's TB elimination efforts by integrating proven preventive measures into routine health system settings. This will help ensure that every TB patient's family and close contacts receive the screening and prophylactic care they need to stay TB-free.

Integrated Lung Health Initiative for Tribal Populations

Donor: Stop TB Partnership

Project iLIFT (Integrated Lung Health Initiative for Tribals) is an initiative aimed at strengthening the early detection of multiple diseases, including TB, chronic respiratory diseases (like chronic obstructive pulmonary disease [COPD] and asthma), hypertension, diabetes, and sickle cell disease, through an integrated approach. The project is being implemented in the Korba district of Chhattisgarh, a region with a large tribal population (around 41%) and over 87,000 mining workers, both of which are high-risk groups for lung and other non-communicable diseases.

The initiative aims to decentralize screening services, improve the diagnostic capacity for respiratory illnesses in public health facilities, and raise community awareness about these conditions. Community-level integrated screening camps will be organized using AI-enabled, ultra-portable chest X-ray units. At these camps, individuals will be screened for a range of conditions: TB, diabetes, hypertension, COPD, asthma, sickle cell disease, and even low body-mass index (BMI, as a marker of undernutrition). All those who screen positive will be referred and actively tracked through diagnosis and initiation of treatment, ensuring no one falls through the cracks.

The goal is to demonstrate a scalable, multi-disease screening model that leverages common resources across traditionally separate health programs. For instance, the project will address gaps in respiratory diagnostics by introducing spirometry (lung function testing) at public facilities for better COPD/asthma diagnosis. Additionally, existing X-ray machines in government facilities will be upgraded with computer-aided detection (CAD) software to enhance the accuracy and efficiency of TB and lung disease diagnosis.

Within one year, Project iLIFT aims to conduct over 250 screening camps, reaching more than 15,000 people. From these efforts, it is expected to identify approximately 50 new TB cases, 750 cases of COPD, 250 cases of asthma, 730 individuals with diabetes, 2,000 with hypertension, and 1,000 sickle cell carriers. By tackling multiple diseases together on a single platform, the project hopes to maximize resource utilization and provide a blueprint for integrated care in other high-need, resource-limited settings.

Market Scoping for Diagnostics and Treatment Products of Drug-Resistant Gonorrhea

Donor: Clinton Health Access Initiative/CARB-X

The global rise of antimicrobial resistance (AMR) is a major threat to public health, particularly in low- and middle-income countries, where diagnostic and surveillance infrastructures are often weak. One concerning pathogen is Neisseria gonorrhoeae—a WHO-designated priority pathogen—which evolves resistance rapidly and is notoriously difficult to culture, hampering both surveillance and effective treatment. This lack of data has long constrained evidence-based decision-making at clinical and public health levels. The continued use of syndromic management (as per WHO guidelines for treating symptomatic sexually transmitted infections) further hampers targeted responses, especially since the majority of gonorrhea infections are asymptomatic (according to WHO and India's National AIDS Control Organisation [NACO]).

Recognizing these challenges, CARB-X—a global non-profit partnership accelerating new antibacterial products—partnered with WJCF to gather insights on drug-resistant gonorrhea. The aim is to inform the development and future deployment of new diagnostics and treatments for this drug-resistant STI.

This initiative is part of a broader effort to enrich CARB-X's product development portfolio with low- and middle-income countries (LMIC)-centric market and health system insights. WJCF's work focused on understanding the current landscape in India: diagnostic capacity for gonorrhea (in both public and private sectors), prevailing treatment protocols, and the key barriers to adopting new tools. The team engaged a wide range of stakeholders, including officials from MoHFW, sexually transmitted infections (STI) specialists, microbiologists, and program managers, to validate the findings and shed light on on-the-ground realities. These discussions revealed issues such as limited availability of gonorrhea diagnostics, minimal case-based management (most cases being treated presumptively), as well as opportunities to introduce new diagnostic, therapeutic, and even preventive products into both public and private healthcare delivery channels. The assessment also underscored the paucity of current data on AMR in gonorrhea in India, highlighting the need for improved surveillance.

As the project progresses, the insights and data gathered will feed into country-specific landscape reports and help shape strategies for product development and market introduction. Ultimately, the findings will support a more evidence-driven approach to gonorrhea and AMR, aiding not only the introduction of new tools in India but also informing global strategies for AMR preparedness.

This collaboration marks an important step toward a more proactive, evidence-based response to drug-resistant gonorrhea in LMICs, with potential lessons for AMR management at large. It also highlights the importance of ensuring that new gonorrhea diagnostics and treatments in the pipeline are aligned with LMIC market needs, healthcare system realities, and regulatory frameworks to maximize their public health impact.

Strengthening India's Response to Lead Poisoning

Donor: Clinton Health Access Initiative

Lead poisoning presents a major public health crisis for India, causing an estimated 6,52,830 premature deaths annually and affecting over 23.3 crore children with blood lead levels exceeding the World Health Organization (WHO) reference value of 5 µg/dL, based on Institute for Health Metrics and Evaluation (IHME) 2021 estimates. WJCF conducted a comprehensive scoping assessment in 2025 to identify targeted interventions to support India's lead poisoning mitigation strategy. At present, WJCF is working on two market shaping interventions:

- Devising market strategies for enabling access to lead-free paints by accelerating the use of safer pigments and curbing reliance on leaded raw materials.
- Accelerating the development of next-generation point-of-care BLL diagnostics by mapping the innovation and supplier landscape, refining the target product profile (TPP), and designing market-shaping incentives to lower barriers and advance product development.

Financial Statement FY 2024-25

William J. Clinton Foundation

Balance Sheet as at 31 March 2025

(All amounts in INR lakh, unless otherwise stated)

	Notes	As at 31 March 2025	As at 31 March 2024
I Sources of Funds			
Restricted funds	3	5,424.50	3,799.83
		5,424.50	**3,799.83**
Non-current liabilities			
Long-term provisions	4	350.17	89.51
		350.17	**89.51**
Current liabilities			
Payables (refer 25A)			
Total outstanding dues of micro enterprises and small enterprises (refer 25B)	5	161.91	90.16
Total outstanding dues other than micro enterprises and small enterprises		86.24	74.38
Other current liabilities	6	184.91	159.47
Short-term provisions	7	35.61	26.58
		468.67	**350.59**
TOTAL		**6,243.34**	**4,239.93**
II Application of Funds			
Non-current assets			
Property, plant and equipment	8	68.91	49.97
Long-term loans and advances	9	70.37	67.03
		139.28	**117.00**
Current assets			
Receivables (refer 25C)	10	200.23	4.58
Cash and bank balances	11	5,864.96	3,923.27
Short-term loans and advances	12	38.87	195.08
		6,104.06	**4,122.93**
TOTAL		**6,243.34**	**4,239.93**

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